# Efficacy and safety of dual antiplatelet therapy in patients undergoing coronary stent implantation: A systematic review and network meta-analysis

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# Supplementary Material

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## Supplementary Table S1 PRISMA checklist

| Section/topic | # | Checklist item | Reported on page # |
|---|---|---|---|
| **TITLE** | | | |
| Title | 1 | Identify the report as a systematic review incorporating a network meta-analysis (or related form of meta-analysis). | 1 |
| **ABSTRACT** | | | |
| Structured summary | 2 | Provide a structured summary including, as applicable: 1) Background: main objectives; 2)Methods: data sources; study eligibility criteria, participants, and interventions; study appraisal; and synthesis methods, such as network meta-analysis; 3) Results: number of studies and participants identified; summary estimates with corresponding confidence/credible intervals; treatment rankings may also be discussed. Authors may choose to summarize pairwise comparisons against a chosen treatment included in their analyses for brevity. 4) Discussion/Conclusions: limitations; conclusions and implications of findings. 5) Other: primary source of funding; systematic review registration number with registry name. | 2 |
| **INTRODUCTION** | | | |
| Rationale | 3 | Describe the rationale for the review in the context of what is already known, including mention of why a network meta-analysis has been conducted. | 3 |
| Objectives | 4 | Provide an explicit statement of questions being addressed with reference to participants, interventions, comparisons, outcomes, and study design (PICOS). | 3 |
| **METHODS** | | | |
| Protocol and registration | 5 | Indicate if a review protocol exists, if and where it can be accessed (e.g., Web address), and, if available, provide registration information including registration number. | N/A |

| | | | |
|---|---|---|---|
| Eligibility criteria | 6 | Specify study characteristics (e.g., PICOS, length of follow-up) and report characteristics (e.g., years considered, language, publication status) used as criteria for eligibility, giving rationale. Clearly describe eligible treatments included in the treatment network, and note whether any have been clustered or merged into the same node (with justification). | 4 |
| Information sources | 7 | Describe all information sources (e.g., databases with dates of coverage, contact with study authors to identify additional studies) in the search and date last searched. | 4 |
| Search | 8 | Present full electronic search strategy for at least one database, including any limits used, such that it could be repeated. | Appendix Table S2 |
| Study selection | 9 | State the process for selecting studies (i.e., screening, eligibility, included in systematic review, and, if applicable, included in the meta-analysis). | 4 |
| Data collection process | 10 | Describe method of data extraction from reports (e.g., piloted forms, independently, in duplicate) and any processes for obtaining and confirming data from investigators. | 4-5 |
| Data items | 11 | List and define all variables for which data were sought (e.g., PICOS, funding sources) and any assumptions and simplifications made. | 4-5 |
| Geometry of the network | S1 | Describe methods used to explore the geometry of the treatment network under study and potential biases related to it. This should include how the evidence base has been graphically summarized for presentation, and what characteristics were compiled and used to describe the evidence base to readers. | 5 |
| Risk of bias within individual studies | 12 | Describe methods used for assessing risk of bias of individual studies (including specification of whether this was done at the study or outcome level), and how this information is to be used in any data synthesis. | 5 |
| Summary measures | 13 | State the principal summary measures (e.g., risk ratio, difference in means). Also describe the use of additional summary measures assessed, such as treatment rankings and surface under the cumulative ranking curve (SUCRA) values, as well as modified approaches used to present summary findings from meta-analyses. | 5 |
| Planned method of analysis | 14 | Describe the methods of handling data and combining results of studies for each network meta-analysis. This should include, but not be limited to: handling of multi-group trials; Selection of variance structure; Selection of prior | 5 |

| | | | |
|---|---|---|---|
| | | distributions in Bayesian analyses; and Assessment of model fit. | |
| Assessment of inconsistency | S2 | Describe the statistical methods used to evaluate the agreement of direct and indirect evidence in the treatment network(s) studied. Describe efforts taken to address its presence when found. | 5 |
| Risk of bias across studies | 15 | Specify any assessment of risk of bias that may affect the cumulative evidence (e.g., publication bias, selective reporting within studies). | 5 |
| Additional analyses | 16 | Describe methods of additional analyses if done, indicating which were pre-specified. This may include, but not be limited to, the following: sensitivity or subgroup analyses; Meta-regression analyses; Alternative formulations of the treatment network; and Use of alternative prior distributions for Bayesian analyses (if applicable). | 5 |
| **RESULTS** | | | |
| Study selection | 17 | Give numbers of studies screened, assessed for eligibility, and included in the review, with reasons for exclusions at each stage, ideally with a flow diagram. | 6 |
| Presentation of network structure | S3 | Provide a network graph of the included studies to enable visualization of the geometry of the treatment network. | Figure 2 |
| Summary of network geometry | S4 | Provide a brief overview of characteristics of the treatment network. This may include commentary on the abundance of trials and randomized patients for the different interventions and pairwise comparisons in the network, gaps of evidence in the treatment network, and potential biases reflected by the network structure. | 6-7 |
| Study characteristics | 18 | For each study, present characteristics for which data were extracted (e.g., study size, PICOS, follow-up period) and provide the citations. | Appendix Table S3, S4, and S5 |
| Risk of bias within studies | 19 | Present data on risk of bias of each study and, if available, any outcome level assessment. | Appendix Table S7 |
| Results of individual studies | 20 | For all outcomes considered (benefits or harms), present, for each study: 1) simple summary data for each intervention group, and 2) effect estimates and confidence intervals. Modified approaches may be needed to deal with information from larger networks. | Appendix Table S6 |

| | | | |
|---|---|---|---|
| Synthesis of results | 21 | Present results of each meta-analysis done, including confidence/credible intervals. In larger networks, authors may focus on comparisons versus a particular comparator (e.g., placebo or standard care), with full findings presented in an appendix. League tables and forest plots may be considered to summarize pairwise comparisons. If additional summary measures were explored (such as treatment rankings), these should also be presented. | 8-10 |
| Exploration of inconsistency | S5 | Describe results from investigations of inconsistency. This may include such information as measures of model fit to compare consistency and inconsistency models, P values from statistical tests, or summary of inconsistency estimates from different parts of the treatment network. | 11 |
| Risk of bias across studies | 22 | Present results of any assessment of risk of bias across studies for the evidence base being studied. | 11 |
| Results of additional analyses | 23 | Give results of additional analyses, if done (e.g., sensitivity or subgroup analyses, meta-regression analyses, alternative network geometries studied, alternative choice of prior distributions for Bayesian analyses, and so forth). | 11-14 |
| **DISCUSSION** | | | |
| Summary of evidence | 24 | Summarize the main findings, including the strength of evidence for each main outcome; consider their relevance to key groups (e.g., health care providers, researchers, and policymakers). | 14-17 |
| Limitations | 25 | Discuss limitations at study and outcome level (e.g., risk of bias), and at review level (e.g., incomplete retrieval of identified research, reporting bias). Comment on the validity of the assumptions, such as transitivity and consistency. Comment on any concerns regarding network geometry (e.g., avoidance of certain comparisons). | 18 |
| Conclusions | 26 | Provide a general interpretation of the results in the context of other evidence, and implications for future research. | 18 |
| **FUNDING** | | | |
| Funding | 27 | Describe sources of funding for the systematic review and other support (e.g., supply of data); role of funders for the systematic review. This should also include information regarding whether funding has been received from manufacturers of treatments in the network and/or whether some of the authors are content experts with professional conflicts of interest that could affect use of treatments in the network. | N/A |

## Supplementary Table S2 Individual search algorithm

| PubMed | search strategies | Results |
|---|---|---|
| #1 | Drug-eluting stent[MeSH Terms] | 11,909 |
| #2 | coronary stent*[Title/Abstract] | 7,328 |
| #3 | #1 OR #2 | 18,004 |
| #4 | Percutaneous Coronary Intervention[MeSH Terms] | 55,427 |
| #5 | (percutaneous coronary[Title/Abstract]) AND revascular*[Title/Abstract] | 9,966 |
| #6 | PCI[Title/Abstract] | 26,763 |
| #7 | #4 OR #5 OR #6 | 67,755 |
| #8 | Platelet Aggregation Inhibitors[MeSH Terms] | 37,171 |
| #9 | antiplatelet*[Title/Abstract] OR anti-platelet*[Title/Abstract] OR antithrombocytic[Title/Abstract] OR anti-thrombocytic[Title/Abstract] OR cyclooxygenase inhibitor*[Title/Abstract] OR thienopyridine*[Title/Abstract] OR thromboxane A2 inhib*[Title/Abstract] OR aspirin[MeSH Terms] OR clopidogrel[Title/Abstract] OR ticagrelor[Title/Abstract] OR prasugrel[Title/Abstract] | 82,739 |
| #10 | #8 OR #9 | 96,044 |
| #11 | #3 OR #7 AND #10 | 10,871 |
| #12 | ((randomized controlled trial[Publication Type]) OR randomized controlled trial[MeSH Terms])OR controlled clinical trial[Publication Type] | 746,412 |
| #13 | (((Animal Experimentation[MeSH Terms]) OR Animals,Laboratory[MeSH Terms]) OR Models, Animal[MeSH Terms]) OR rodentia[MeSH Terms] | 3,418,355 |
| #14 | #11 AND #12 NOT #13 | 2,480 |
| **Embase** | **search strategies** | **Results** |
| #1 | 'drug-eluting stent'/exp | 33,827 |
| #2 | coronary:ab,ti AND stent*:ab,ti | 51,190 |
| #3 | #1 OR #2 | 66,566 |
| #4 | percutaneous coronary intervention'/exp | 102,108 |

| | | |
|---|---|---|
| #5 | percutaneous AND coronary AND revascular*:ab,ti | 24,709 |
| #6 | pci:ab,ti | 57,812 |
| #7 | #4 OR #5 OR #6 | 126,939 |
| #8 | 'antithrombocytic agent'/exp | 352,634 |
| #9 | antiplatelet*:ab,ti OR 'anti platelet*':ab,ti OR antithrombocytic:ab,ti OR 'anti thrombocytic':ab,ti OR (cyclooxygenase:ab,ti AND inhibitor*:ab,ti) OR thienopyridine*:ab,ti OR (thromboxane:ab,ti AND a2:ab,ti AND inhib*:ab,ti) OR 'aspirin'/exp OR clopidogrel:ab,ti OR ticagrelor:ab,ti OR prasugrel:ab,ti | 278,996 |
| #10 | #8 OR #9 | 393,556 |
| #11 | andomized controlled trial' OR 'randomized controlled trials as topic'/exp OR random*:ti,ab | 1,639,922 |
| #12 | 'animals, laboratory'/exp OR 'animal experiment'/exp OR 'animal model'/exp OR 'rodent'/exp | 4,635,855 |
| #13 | (#3 OR #7) AND #10 AND #11 NOT #12 | 8,341 |
| **Cochrane Library** | **search strategies** | **Results** |
| #1 | [mh "drug-eluting stents"] | 1,423 |
| #2 | coronary stent*:ti,ab,kw | 8527 |
| #3 | #1 OR #2 | 8606 |
| #4 | [mh "Percutaneous Coronary Intervention"] | 5567 |
| #5 | percutaneous coronary revascular*:ti,ab,kw | 4204 |
| #6 | PCI:ti,ab,kw | 8366 |
| #7 | #4 OR #5 OR #6 | 12846 |
| #8 | [mh "Platelet aggregation inhibitors"] OR [mh "Cyclooxygenase Inhibitors"] OR [mh Thienopyridines] OR [mh "Thromboxane A2"/AI] OR [mh "Purinergic P2Y Receptor Antagonists"] | 6264 |
| #9 | antiplatelet* OR anti-platelet* OR antithrombocytic OR anti-thrombocytic OR clopidogrel or ticagrelor or prasugrel OR aspirin*:ti,ab,kw | 20109 |
| #10 | #8 OR #9 | 22318 |
| #11 | randomized OR clinical trials:ti,ab,kw | 1128131 |
| #12 | rat OR rats OR mouse OR mice:ti,ab,kw OR [mh animals] | 589756 |
| #13 | (#3 OR #7)AND #10 AND #11 NOT #12 | 1698 |

| Web of Science | search strategies | Results |
|---|---|---|
| #1 | TS=drug-eluting stent | 13626 |
| #2 | TS=coronary stent* | 43772 |
| #3 | #1 OR #2 | 47275 |
| #4 | TS=Percutaneous Coronary Intervention | 54290 |
| #5 | TS=percutaneous coronary revascular* | 15203 |
| #6 | TS=PCI | 35542 |
| #7 | #4 OR #5 OR #6 | 72283 |
| #8 | TS=Platelet aggregation inhibitors | 9529 |
| #9 | TS=(antiplatelet*OR anti-platelet* OR antithrombocytic OR anti-thrombocytic OR cyclooxygenase inhibitor* OR thienopyridine* OR thromboxane A2 inhibitor* OR aspirin OR clopidogrel OR ticagrelor OR prasugrel) | 103258 |
| #10 | #8 OR #9 | 110089 |
| #11 | TS=(clinical trial* OR controlled trial* OR random* trial* OR random*stud*) | 1210556 |
| #12 | (#3 OR #7) AND #10 AND #11 | 3614 |
| #13 | (#3 OR #7) AND #10 AND #11 limit article | 2678 |
| **ClinicalTrials.gov** | | |
| 379 Studies found for: Antiplatelet Drug OR Antiplatelet Aggregation Inhibitors \| Interventional Studies \| PCI OR Percutaneous Coronary Intervention OR drug-eluting stent | | |
| **Unpublished evidence** | | |
| we used following keywords in different combinations to search unpublished trials at relevant meetings (American Heart Association, American College of Cardiology, European Society of Cardiology): antiplatelet, aspirin, drug eluting stent, percutaneous coronary intervention, from 2018 to 2020. | | |

## Supplementary Table S3 Baseline characteristics of the included trials

| Trials | Randomization | Stent type | Follow-up | Primary endpoint | Treatment strategy |
|---|---|---|---|---|---|
| **TICO** | At index PCI | BP-SES | 12months | Composite of major bleeding and adverse cardiac and cerebrovascular events | DAPT 3Mo+P2Y12: ticagrelor (90 mg twice daily,12months) + aspirin (100 mg daily,3months);<br>DAPT 12Mo: ticagrelor (90 mg twice daily,12months) + aspirin (100 mg daily,12months) |
| **STOPDAPT-2** | At index PCI | EES | 12months | Composite of cardiovascular death, MI, definite ST, ischemic or hemorrhagic stroke, or TIMI major or minor bleeding | DAPT 1Mo+P2Y12: aspirin (81-200mg daily dose,1months) + clopidogrel (75mg daily dose,1months) or prasugrel (3.75mg daily dose,1months) and followed by clopidogrel monotherapy for up to 5 years;<br>DAPT 12Mo: aspirin (81-200mg daily dose,12 months) + clopidogrel (75 mg daily dose,12months) or prasugrel (3.75 mg daily dose,12months) and followed by aspirin monotherapy for up to 5 years |

| | | | | | |
|---|---|---|---|---|---|
| **TWILIGHT** | At 3 months after PCI | DES | 12months | The first occurrence of BARC type 2, 3, or 5 bleeding | DAPT 3Mo+P2Y12: ticagrelor (90 mg twice daily) and aspirin (81-100 mg daily) for 3 months. Then switched saspirin to placebo for an additional 12 months along with continuation ticagrelor treatment;<br>DAPT 12Mo: ticagrelor (90 mg twice daily,15months) + aspirin (81-100 mg daily,15 months) |
| **SMART-CHOICE** | At the index or within 3 months after the index PCI | EES/BP-SES/ZES | 12months | Composite of all-cause death, MI, or stroke | DAPT 3Mo+P2Y12: aspirin (100mg daily dose, 3months) + clopidogrel (75 mg daily dose, indefinitely) or prasugrel (10 mg daily dose, indefinitely) or ticagrelor (90 mg twice daily, indefinitely);<br>DAPT 12Mo: Aspirin (100mg daily dose, indefinitely) + clopidogrel (75 mg daily dose, indefinitely) or prasugrel (10 mg daily dose, indefinitely) or ticagrelor (90 mg twice daily, indefinitely) |
| **REDUCE** | At index PCI | MASCOT | 24months | Composite occurrence of all-cause death, MI, ST, stroke, TVR and bleeding | DAPT 3Mo+ASA :Aspirin (indefinitely)+Pras-ugrel (10 mg daily dose,3months) or Ticagrelor (180 mg daily dose,3months) or Clopidogrel (75mg daily dose,3months);<br>DAPT 12Mo: Aspirin (indefinitely)+Prasu-grel (10 mg daily dose,12months) or Ticagrelor (180 mg daily dose,12months) or Clopidogrel (75mg daily dose,12m-onths) |

| | | | | | |
|---|---|---|---|---|---|
| **GLOBAL LEADERS** | At index PCI | BES | 24months | Composite of all-cause death or new Q-wave MI | DAPT 1Mo+P2Y12:75-100 mg aspirin daily in combination with 90 mg ticagrelor twice daily for 1 month, followed by 90 mg ticagrelor twice daily for 23 months;<br>DAPT 12Mo:75-100 mg aspirin daily in combination with either 75 mg clopidogrel daily (for patients with stable coronary artery disease) or 90 mg ticagrelor twice daily (for patients with acute coronary syndromes), followed by 75-100 mg aspirin daily for 12 months |
| **OPTIMA-C** | At index PCI | BES/ZES | 12months | Composite of cardiac death, target vessel-related MI, ischemia-driven TLR | DAPT 6Mo: aspirin (100 mg daily dose, indefinitely) + clopidogrel (75 mg daily dose, 6 months) ;<br>DAPT 12Mo: aspirin (100 mg daily dose, indefinitely) + clopidogrel (75 mg daily dose, 12 months) |
| **DAPT-STEMI** | At 6 months after PCI | BES/EES/ZES | 24months | Composite of all-cause mortality, any MI, any revascularization, stroke, major bleeding | DAPT 6Mo: aspirin (75-100mg daily dose, indefinitely) +Prasugrel (10 mg daily dose,6months) or Ticagrelor (90 mg twice daily,6months) or Clopidogrel (75mg daily dose,6months);<br>DAPT 12Mo: aspirin (75-100mg daily dose, indefinitely) + Prasugrel (10 mg daily dose,12months) or Ticagrelor (90 mg twice daily,12months) or Clopidogrel (75mg daily dose,12months) |
| **SMART-DATE** | At index PCI | EES/ZES/BES | 18months | Composite of all-cause mortality, MI or stroke | DAPT 6Mo: aspirin (100mg daily dose, indefinitely) + clopidogrel (75mg daily dose ,6months) or prasugrel (10 mg daily dose ,6months) or ticagrelor (90mg twice per day,6months);<br>L-DAPT: aspirin (100mg daily dose, indefinitely) + clopidogrel (75mg daily dose ,12 months or longer) or |

| | | | | | |
|---|---|---|---|---|---|
| | | | | | prasugrel (10 mg daily dose ,12 months or longer) or ticagrelor (90mg twice per day,12 months or longer) |
| **NIPPON** | At index PCI | BES | 18months | Composite of all cause death, Q-wave or non-Q-wave MI, cerebrovascular events, and major bleeding events | L-DAPT: aspirin (81-162mg daily dose, indefinitely) + clopidogrel (75mg daily dose,18 months) /ticlopidine (200mg daily dose,18months);<br>DAPT 6Mo: aspirin (81-162mg daily dose, indefinitely) + clopidogrel (75mg daily dose,6 months) /ticlopidine (200mg daily dose,6 months). |
| **IVUS-XPL** | At index PCI | EES | 12months | Composite of cardiac death, MI, stroke, or TIMI major bleeding | DAPT 6Mo: aspirin (100 mg daily dose, indefinitely) + clopidogrel (75 mg daily dose, 6 months);<br>DAPT 12Mo: aspirin (100 mg daily dose, indefinitely) + clopidogrel (75 mg daily dose, 12 months) |
| **OPTIDUAL** | 12±3 months after PCI | SES/PES/ZES/ EES | 36months | Composite of all-cause mortality, non-fatal MI, stroke, or major bleeding | L-DAPT: aspirin (75-160mg daily dose, indefinitely) + clopidogrel (75mg daily dose,48 months);<br>DAPT 12Mo: aspirin (75-160mg daily dose, indefinitely) + clopidogrel (75mg daily dose,12months) |
| **I-LOVE-IT 2** | At index PCI | BP-SES | 18months | Composite of cardiac death, target vessel MI, or clinically indicated TLV | DAPT 6Mo: aspirin (100 mg daily dose, indefinitely) + clopidogrel (75 mg daily dose, 6 months);<br>DAPT 12Mo: aspirin (100 mg daily dose, indefinitely) + clopidogrel (75 mg daily dose, 12 months) |
| **ISAR-SAFE** | At 6 (-1/+2) months after DES implantation | PES/SES/EES/ ZES/BES/BMS | 15months | Composite of death, myocardial infarction, ST (definite or probable), stroke, or thrombolysis in MI major bleeding | DAPT 6Mo: aspirin (81-200mg daily dose, indefinitely) +clopidogrel (75 mg daily dose, 6 months) and clopidogrel was swithched to placebo after 6 months;<br>DAPT 12Mo: aspirin (81-200 mg daily dose, indefinitely) + clopidogrel (75 mg daily dose, 12months) |

| | | | | | |
|---|---|---|---|---|---|
| **ITALIC** | 6 months after PCI | EES | 24months | Composite of all-cause mortality, MI, TVR, stroke, or major bleeding according to the TIMI criteria | DAPT 6Mo: aspirin (indefinitely) + clopidogrel (75mg daily dose, 6 months) or prasugrel (10mg daily dose, 6 months) or ticagrelor (90mg twice per day, 6months);<br>L-DAPT: aspirin (indefinitely) + clopid-ogrel (75mg daily dose,24 months) or prasugrel (10mg daily dose,24months) or ticagrelor (90mg twice per day,24 months) |
| **DAPT Study** | 12 months after PCI | EES/PES/ZES/ SES | 33months | Composite of definite or probable ST and of major adverse cardiovascular and cerebrovascular events | L-DAPT: aspirin (75-162mg daily dose, indefinitely) + clopidogrel (75mg daily dose, 30months) or prasugrel (10 or 5 mg daily dose,30months);<br>DAPT 12Mo: Aspirin (75-162mg daily dose, indefinitely) + clopidogrel (75 mg daily dose, 12 months) or prasugrel (10 or 5mg daily dose, 12 months) continued by placebo |
| **DES LATE** | 12 to 18 months after PCI | SES/PES/ZES/ EES/others | Median length of follow-up was 42.0 months (interquartile range, 24.7-50.7 months) | Composite of death resulting from cardiac causes, MI, or stroke 24 months after randomization. | DAPT 12Mo: aspirin (100-200mg daily dose, indefinitely) + clopidogrel (75mg daily dose,12 months);<br>L-DAPT: aspirin (100-200mg daily dose, indefinitely) + clopidogrel (75mg daily dose, 36 months) |
| **SECURITY** | At index PCI | BES/EES/ZES | 24months | Composite of cardiac death, MI, stroke, definite or probable stent thrombosis, or BARC criteria type 3 or 5 bleeding | DAPT 6Mo: aspirin (indefinitely) + clopidogrel (75 mg daily dose, 6 months);<br>DAPT 12Mo: aspirin (indefinitely) +clopidogrel (75 mg daily dose, 12 months) |
| **ARCTIC-Interruption** | 12 months after PCI | First or second-generation DES | Median duration of follow-up was 17 months (interquartile | Composite of death, myocardial infarction, stent thrombosis, stroke, or urgent revascularisation | L-DAPT: aspirin (indefinitely) + clopidogrel (75-150mg daily dose, 18-30months) or purasugrel (10 mg daily dose, 18-30months);<br>DAPT12Mo: aspirin (indefinitely) + clopidogrel (75-150mg |

| | | | | | |
|---|---|---|---|---|---|
| | | | range,15 to 18) | | daily dose, 12months) or purasugrel (10 mg daily dose, 12months) |
| **OPTIMIZE** | At index PCI | ZES | 12months | Composite of all-cause mortality, MI, stroke, or major bleeding | DAPT 3Mo+ASA: aspirin (100-200mg daily dose, indefinitely) + clopidogrel (75 mg daily dose,3 months);<br>DAPT 12Mo: aspirin (100-200mg daily dose,indefinitely) + clopidogrel (75 mg daily dose, 12 months) |
| **PRODIGY** | 30±5 days after PCI | EES/PES/ZES/ BMS | 24months | Composite of death of any cause, MI, or cerebrovascular accident | L-DAPT: aspirin (80-160mg daily dose, indefinitely) + clopidogrel (75 mg daily dose, 24 months);<br>DAPT 6Mo: aspirin (80-160mg daily dose, indefinitely) + clopidogrel (75 mg daily dose, 6 months) |
| **RESET** | At index PCI | ZES/SES/EES | 12months | Composite of death from cardiovascular cause, MI, ST, ischemia-driven TVR, or bleeding | DAPT 3Mo+ASA: aspirin (100mg daily dose, indefinitely) + clopidogrel (75 mg daily dose,3months);<br>DAPT 12Mo: aspirin (100mg daily dose, indefinitely) + clopidogrel (75 mg daily dose,12months) |
| **EXCELLENT** | At index PCI | EES/SES | 12months | Composite of cardiac death, MI, or TVR | DAPT 6Mo: aspirin (100-200mg daily dose, 12months) + clopidogrel (75 mg daily dose,6 months);<br>DAPT12Mo: aspirin (100-200mg daily dose, 12months) + clopidogrel (75 mg daily dose, 12 months) |
| **REAL-ZEST LATE** | 12 months after PCI | SES/PES/ZES/ other | Median duration of follow-up was 19.2 months (interquartile range, 13.2 to 24.1) | The first occurrence of MI or death from cardiac causes after assignment to a treatment group | L-DAPT: aspirin (100-200mg daily dose, indefinitely) + clopidogrel (75 mg daily dose, 36months);<br>DAPT 12Mo: aspirin (100-200mg daily dose, indefinitely) + clopidogrel (75 mg daily dose,12 months) |

PCI, percutaneous coronary intervention; MI, myocardial infarction; ST, stent thrombosis; DAPT, dual antiplatelet therapy; L-DAPT, longer than 12 months DAPT; DAPT 12Mo,12

months DAPT; S-DAPT+ASA, shorter than 6-month DAPT followed by aspirin monotherapy; S-DAPT+P2Y12, shorter than 6-month DAPT followed by a P2Y12 receptor inhibitor monotherapy; TIMI, thrombolysis in MI; BARC, Bleeding Academic Research Consortium; BES, biolimus-eluting stent ; ZES, zotarolimus-eluting stent; BP-SES, biodegradable polymer sirolimus-eluting stent; EES, everolimus-eluting stent; PES, paclitaxel-eluting stent; SES, sirolimus-eluting stent; MASCOT, The Multinational Abluminal Sirolimus Coated Bio-engineered Stent ;TLR, target lesion revascularization; TVR, target vessel revascularization; NSTEMI, non-ST elevation myocardial infarction; STEMI, ST elevation myocardial infarction;

# Supplementary Table S4 Baseline characteristics of participants

| Characteristics | TICO | | STOPDAPT-2 | | TWILIGHT | | SMART-CHOICE | |
|---|---|---|---|---|---|---|---|---|
| | DAPT 3Mo+ P2Y12 | DAPT 12Mo | DAPT 1Mo+ P2Y12 | DAPT 12Mo | DAPT 3Mo+ P2Y12 | DAPT 12Mo | DAPT 3Mo+ P2Y12 | DAPT 12Mo |
| No. of participants | 1527 | 1529 | 1500 | 1509 | 3555 | 3564 | 1495 | 1498 |
| Age, mean(SD), years | 61 (11) | 61 (11) | 68.1 (10.9) | 69.1 (10.4) | 65.2±10.3 | 65.1±10.4 | 64.6 (10.7) | 64.4 (10.7) |
| Male, n/ total (%) | 1204(79) | 1224(80) | 1183 (78.9) | 1154 (76.5) | 2709(76.2) | 2712(76.1) | 1087 (72.7) | 1111 (74.2) |
| BMI, mean(SD), $kg/m^2$ | 24.9 (3.2) | 24.9 (3.3) | 24.4 (3.5) | 24.2 (3.5) | 28.6±5.5 | 28.5±5.6 | 24.5 (3.1) | 24.7 (3.2) |
| Cardiovascular risk factors, n/total (%) | | | | | | | | |
| Hypertension | 760 (50) | 781 (51) | 1105 (73.7) | 1116 (74.0) | 2580 (72.6) | 2574 (72.2) | 921 (61.6) | 919 (61.3) |
| Diabetes | 418 (27) | 417 (27) | 585 (39.0) | 574 (38.0) | 1319(37.1) | 1301(36.5) | 570 (38.2) | 552 (36.8) |
| Dyslipidemia | 924 (61) | 922 (60) | 1116 (74.4) | 1128 (74.8) | 2157 (60.7) | 2146 (60.2) | 673 (45.1) | 679 (45.5) |
| Current smoker | 555 (36) | 587 (38) | 399 (26.6) | 311 (20.6) | 726 (20.4) | 822 (23.1) | 424 (28.4) | 367 (24.5) |
| History, n/ total (%) | | | | | | | | |
| MI | 64 (4) | 49 (3) | 207 (13.8) | 199 (13.2) | 1020(28.7) | 1020(28.6) | 62 (4.1) | 65 (4.3) |
| PCI | 135 (9) | 127 (8) | 503 (33.5) | 529 (35.1) | 1502 (42.3) | 1496 (42.0) | 172 (11.5) | 177 (11.8) |
| Coronary artery bypass graft | 8 (1) | 10 (1) | 17 (1.1) | 42 (2.8) | 362 (10.2) | 348 (9.8) | NA | NA |
| Stroke | 60 (4) | 66 (4) | 81 (5.4) | 105 (7.0) | NA | NA | 99 (6.6) | 102 (6.8) |
| Clinical presentation, n/ total (%) | | | | | | | | |
| Stable angina | NA | NA | 935 (62.3) | 926 (61.4) | 1047 (29.5) | 999 (28.0) | 625 (41.8) | 625 (41.8) |
| UA | 442 (29) | 484 (32) | 193 (12.9) | 214 (14.2) | 1249 (35.1) | 1245(34.9) | 467 (31.2) | 491 (32.8) |
| NSTEMA | 539 (35) | 488 (32) | 81 (5.4) | 99 (6.6) | 1024 (28.8) | 1096 (30.8) | 239 (16.0) | 230 (15.4) |
| STEMA | 546 (36) | 557 (36) | 291 (19.4) | 270 (17.9) | NA | NA | 164 (11.0) | 150 (10.0) |
| Target lesion site, n (%) | | | | | | | | |
| Left main | 49 (3) | 45 (2) | 43 (2.9) | 37 (2.5) | 166 (4.7) | 187 (5.2) | 23 (1.2) | 35 (1.9) |
| Left anterior descending artery | 912 (48) | 909 (48) | 828 (55.2) | 854 (56.6) | 1993 (56.1) | 2010 (56.4) | 903 (48.8) | 950 (50.4) |
| Left circumflex artery | 358 (19) | 353 (19) | 268 (17.9) | 305 (20.2) | 1151 (32.4) | 1146 (32.2) | 399 (21.6) | 376 (19.9) |
| Right coronary artery | 565 (30) | 588 (31) | 436 (29.1) | 410 (27.2) | 1243 (35.0) | 1257 (35.3) | 524 (28.3) | 524 (27.8) |

| Characteristics | REDUCE | | GLOBAL LEADERS | | OPTIMA-C | | DAPT-STEMI | |
|---|---|---|---|---|---|---|---|---|
| | DAPT 3Mo+ ASA | DAPT 12Mo | DAPT 1Mo+ P2Y12 | DAPT 12Mo | DAPT 6Mo | DAPT 12Mo | DAPT 6Mo | DAPT 12Mo |
| No. of participants | 751 | 745 | 7980 | 7988 | 683 | 684 | 433 | 437 |
| Age, mean(SD), years | 61.0 | 60.0 | 64.5(10.3) | 64.6(10.3) | 62.8±10.8 | 64.4±10.3 | 59.8±10.7 | 60.2±10.3 |
| Male, n/ total (%) | 620(82.6) | 576(77.3) | 6115 (76.6) | 6139(76.9) | 478 (70.0) | 464 (67.8) | 337 (78) | 332 (76) |
| BMI, mean(SD), $kg/m^2$ | 26.6 | 26.6 | 28.2 (4.6) | 28.2 (4.6) | NA | NA | 27.8±4.3 | 27.9±4.5 |
| Cardiovascular risk factors, n/total (%) | | | | | | | | |
| Hypertension | 379(50.7) | 375(50.7) | 5882(74.0) | 5833(73.3) | 426 (62.4) | 437 (63.9) | 193 (45) | 195(45) |
| Diabetes | 162(21.6) | 145(19.5) | 2049(25.7) | 1989(24.9) | 199 (29.1) | 203 (29.7) | 54 (13) | 61(14) |
| Dyslipidemia | 346(46.3) | 333(44.9) | 5345(69.3) | 5423(70.0) | 204 (29.9) | 195 (28.5) | 120 (28) | 125(29) |
| Current smoker | 313(42.1) | 314(42.7) | 2066(25.9) | 2103(26.3) | 184 (26.9) | 184 (26.9) | 218(51) | 205(47) |
| History, n/ total (%) | | | | | | | | |
| MI | NA | NA | 1831(23.0) | 1879(23.6) | 18 (2.6) | 25 (3.7) | 26 (6) | 20(5) |
| PCI | 88(11.7) | 73(9.8) | 2609 (32.7) | 2612(32.7) | 59 (8.6) | 71 (10.4) | 29 (7) | 18(4) |
| Coronary artery bypass graft | 21(2.8) | 21(2.8) | 448(5.6) | 495(6.2) | NA | NA | 8 (2) | 2(0.5) |
| Stroke | 11(1.5) | 15(2.0) | 210(2.6) | 211(2.6) | NA | NA | 14 (3) | 8(2) |
| Clinical presentation, n/ total (%) | | | | | | | | |
| Stable angina | NA | NA | 4230(53.0) | 4251(53.2) | 339 (49.6) | 336 (49.1) | NA | NA |
| UA | 114(15.2) | 103(13.8) | 1004(12.6) | 1018(12.7) | 254 (37.2) | 253 (37.0) | NA | NA |
| NSTEMA | 267(35.6) | 305(41.0) | 1684(21.1) | 1689(21.1) | 90 (13.2) | 95 (13.9) | NA | NA |
| STEMA | 370 (49.3) | 336(45.2) | 1062(13.3) | 1030(12.9) | NA | NA | NA | NA |
| Target lesion site, n (%) | | | | | | | | |
| Left main | NA | NA | 197(1.9) | 190 (1.8) | NA | NA | NA | NA |
| Left anterior descending artery | 360(48) | 329(44.2) | 4283(41.2) | 4383 (42.0) | 394(57.7) | 353(51.6) | 169 (39) | 188(43) |
| Left circumflex artery | 146 (19.5) | 164(22.0) | 2524(24.3) | 2553 (24.5) | 134(19.6) | 165(24.1) | 89 (21) | 70(16) |
| Right coronary artery | 234(31.2) | 246(33.0) | 3284 (31.6) | 3206 (30.7) | 155(22.7) | 166(24.3) | 175 (41) | 179(41) |

| Characteristics | SMART-DATE | | NIPPON | | IVUS-XPL | | OPTIDUAL | |
|---|---|---|---|---|---|---|---|---|
| | DAPT 6Mo | L-DAPT | L-DAPT | DAPT 6Mo | DAPT 6Mo | DAPT 12Mo | L-DAPT | DAPT 12Mo |
| No. of participants | 1357 | 1355 | 1653 | 1654 | 699 | 701 | 695 | 690 |
| Age, mean(SD), years | 62(54-71) | 63(53-71) | 67.2±9.9 | 67.4±9.6 | 63±9 | 64±9 | 64.1±10.8 | 64.2 ±11.5 |
| Male, n/ total (%) | 1016(74.9) | 1028 (75.9) | 1312(79.4) | 1,304(78.8) | 470(67) | 494(70) | 568(81.7) | 547(79.3) |
| BMI, mean(SD), $kg/m^2$ | 24.3(3.2) | 24.5(3.1) | 24.3±3.5 | 24.5±3.5 | 24.8±3.1 | 24.6±3.0 | NA | NA |
| Cardiovascular risk factors, n/total (%) | | | | | | | | |
| Hypertension | 669(49.9) | 654(48.7) | 1209 (73.1) | 1177(71.2) | 443(63) | 455(65) | 396(57.0) | 417(60.4) |
| Diabetes | 365(26.9) | 379(28.1) | 635(38.4) | 619(37.4) | 249(36) | 257(37) | 213(30.6) | 222(32.2) |
| Dyslipidemia | 322(24.2) | 336(25.2) | 1132(68.5) | 1130(68.3) | 473(68) | 456(65) | NA | NA |
| Current smoker | 506(38.0) | 536(40.1) | 997(60.3) | 960(58.0) | 171(25) | 165(24) | 425(61.2) | 399(57.8) |
| History, n/ total (%) | | | | | | | | |
| MI | 30(2.3) | 23(1.7) | 195(11.8) | 201(12.2) | 34(5) | 29(4) | 119(17.1) | 122(17.7) |
| PCI | 65(4.9) | 73(9.8) | 432(26.1) | 413(25.0) | 72(10) | 73(10) | 180(25.9) | 186(27.0) |
| Coronary artery bypass graft | NA | NA | 29(1.8) | 22(1.3) | 22(3) | 14(2) | 37(5.3) | 35(5.1) |
| Stroke | 52(3.9) | 58(4.4) | 41(2.5) | 48(2.9) | NA | NA | 29(4.2) | 25(3.6) |
| Clinical presentation, n/ total (%) | | | | | | | | |
| Stable angina | NA | NA | 734(44.4) | 805(48.7) | 356(51) | 358(51) | 240(34.5) | 207(30.0) |
| UA | 420 (31.0) | 416(30.7) | 330(20.0) | 296(17.9) | 237(34) | 231(31) | 66(9.5) | 63(9.1) |
| NSTEMA | 428 (31.5) | 425(31.4) | 26(1.6) | 33(2.0) | NA | NA | 99(14.2) | 117(17.0) |
| STEMA | 509 (37.5) | 514(37.9) | 196(11.9) | 198(12.0) | NA | NA | 74(10.7) | 82(11.9) |
| Target lesion site, n (%) | | | | | | | | |
| Left main | 29(2.1) | 17(1.3) | 16(0.8) | 7(0.4) | NA | NA | 4(<1) | 2(<1) |
| Left anterior descending artery | 767(56.6) | 826(61.0) | 998(52.4) | 981(51.8) | 518(55) | 530(56) | 397(57) | 443(64) |
| Left circumflex artery | 331(24.4) | 340(25.1) | 374(19.7) | 381(20.1) | 187(20) | 175(18) | 225(32) | 214(31) |
| Right coronary artery | 504(37.2) | 490(36.2) | 515(27.1) | 524(27.7) | 239(25) | 245(26) | 280 (40) | 268(39) |

| Characteristics | I-LOVE-IT 2 | | ISAR-SAFE | | ITALIC | | DAPT Study | |
|---|---|---|---|---|---|---|---|---|
| | DAPT 6Mo | DAPT 12Mo | DAPT 6Mo | DAPT 12Mo | DAPT 6Mo | L-DAPT | L-DAPT | DAPT 12Mo |
| No. of participants | 909 | 920 | 1997 | 2003 | 926 | 924 | 5020 | 4941 |
| Age, mean(SD), years | 60.4±10.2 | 60.0±10.0 | 67.2(59.3–73.3) | 67.2(59.1–73.7) | 61.6±10.9 | 61.5±11.2 | 61.8±10.2 | 61.6±10.1 |
| Male, n/ total (%) | 611(67.2) | 632(68.7) | 1611(80.7) | 1612(80.5) | 750(81.0) | 733(79.3) | 3778(75.3) | 3657(74.0) |
| BMI, mean(SD), $kg/m^2$ | 25.1±3.1 | 25.1±3.1 | 27.2(24.9–30.1) | 27.5(24.9–30.4) | 27.0±4.6 | 227.1±4.7 | 30.5±5.8 | 30.6±5.8 |
| Cardiovascular risk factors, n/total (%) | | | | | | | | |
| Hypertension | 554(61.0) | 596(64.8) | 1797(90.1) | 1830(91.5) | 603(65.1) | 594(64.3) | 3796(75.8) | 3649(74.0) |
| Diabetes | 211(23.2) | 203(22.1) | 495(24.8) | 484(24.2) | 336(36.3) | 349(37.8) | 1556(31.1) | 1481(30.1) |
| Dyslipidemia | 230(25.3) | 215(23.4) | 1747(87.5) | 1748(87.4) | 625(67.5) | 618(66.9) | NA | NA |
| Current smoker | 333(36.6) | 5352(38.3) | 292(14.6) | 306(15.3) | 473(51.1) | 487(52.7) | 1222(24.62) | 1210(24.7) |
| History, n/ total (%) | | | | | | | | |
| MI | 156(17.2) | 145(15.8) | 516(25.9) | 491(24.5) | 144(15.6) | 138(14.9) | 1092(22.0) | 1026(21.1) |
| PCI | 77(8.5) | 60(6.5) | NA | NA | 226(24.4) | 209(22.6) | 1518(30.4) | 1529(31.0) |
| Coronary artery bypass graft | 4(0.4) | 4(0.4) | 152(7.7) | 149(7.5) | 61(6.6) | 45(4.9) | 568(11.3) | 581(11.8) |
| Stroke | 84(9.2) | 87(9.5) | NA | NA | 28(3.0) | 26(2.8) | 155(3.1) | 169(3.4) |
| Clinical presentation, n/ total (%) | | | | | | | | |
| Stable angina | 130(14.3) | 139(15.1) | 969(48.6) | 956(47.8) | 382(41.3) | 383(41.5) | 1882(37.5) | 1870(37.8) |
| UA | 527(58.0) | 520(56.5) | 429(21.5) | 438(21.9) | 188(20.3) | 186(20.1) | 838(16.7) | 825(16.7) |
| NSTEMA | 103(11.3) | 98(10.7) | 207(10.4) | 203(10.1) | 145(15.7) | 152(16.5) | 776(15.5) | 767(15.5) |
| STEMA | 122(13.4) | 126(13.7) | 158(7.9) | 166(8.3) | 67(7.2) | 68(7.4) | 534(10.6) | 511(10.3) |
| Target lesion site, n (%) | | | | | | | | |
| Left main | 23(1.9) | 21(1.7) | 9(0.5) | 3(0.2) | 14(1.5) | 8(0.9) | 55(0.8) | 55(0.9) |
| Left anterior descending artery | 569(45.9) | 569(45.3) | 794(39.8) | 812(40.6) | 669(73.4) | 658(72.3) | 2715(41.2) | 2586(40.4) |
| Left circumflex artery | 284(22.9) | 279(22.2) | 528(26.4) | 480(24.0) | 456(50.0) | 436(47.9) | 1473(22.4) | 1506(23.5) |
| Right coronary artery | 364(29.4) | 386(30.8) | 636(31.8) | 682(34.0) | 489(53.6) | 474(52.1) | 2153(32.7) | 2057(32.1) |

| Characteristics | DES LATE | | SECURITY | | ARCTIC-Interruption | | OPTIMIZE | |
|---|---|---|---|---|---|---|---|---|
| | DAPT 12Mo | L-DAPT | DAPT 6Mo | DAPT 12Mo | L-DAPT | DAPT 12Mo | DAPT 3Mo+ ASA | DAPT 12Mo |
| No. of participants | 2514 | 2531 | 682 | 717 | 636 | 624 | 1563 | 1556 |
| Age, mean(SD), years | 62.3±10.1 | 62.5±10.0 | 64.9±10.2 | 65.5±10.1 | 64(57–73) | 64(57–73) | 61.3(10.4) | 61.9(10.6) |
| Male, n/ total (%) | 1749(69.6) | 1749(69.1) | 529(77.6) | 551(76.8) | 508(80) | 503(81) | 992(63.5) | 982(63.1) |
| BMI, mean(SD), $kg/m^2$ | NA | NA | NA | NA | 27(25-29) | 27(25-30) | NA | NA |
| Cardiovascular risk factors, n/total (%) | | | | | | | | |
| Hypertension | 1423(56.6) | 1479(58.4) | 508(74.5) | 510(71.1) | 376(59) | 388(62) | 1350(86.4) | 1371(88.2) |
| Diabetes | 709(28.2) | 709(28.0) | 206(30.4) | 223(31.7) | 222(36) | 349(37.8) | 554(35.4) | 549(35.3) |
| Dyslipidemia | NA | NA | 446(65.4) | 436(60.8) | 428(67) | 426(68) | 953(63.2) | 952(63.7) |
| Current smoker | 722(28.7) | 693(27.4) | 139(20.5) | 172(24.4) | 147(23) | 152(24) | 290(18.6) | 269(17.3) |
| History, n/ total (%) | | | | | | | | |
| MI | 92(3.7) | 103(4.1) | 145(21.2) | 144(20.1) | 197(31) | 186(30) | 541(34.6) | 542(34.8) |
| PCI | 276(11.0) | 313(12.4) | 132(19.4) | 116(16.2) | 273(43) | 249(40) | 327(20.9) | 297(19.1) |
| Coronary artery bypass graft | NA | NA | 38(5.6) | 39(5.4) | 47(7) | 35(6) | 111(7.1) | 128(8.2) |
| Stroke | 89(3.5) | 15(4.5) | NA | NA | 28(4) | 38(6) | NA | NA |
| Clinical presenta-tion, n/ total (%) | | | | | | | | |
| Stable angina | 956(38.0) | 1011(39.9) | 341(61.6) | 368(61.6) | NA | NA | 935(59.8) | 911(58.6) |
| UA | 971(38.6) | 930(36.7) | 213(38.4) | 229(38.4) | NA | NA | NA | NA |
| NSTEMA | 266(10.6) | 268(10.6) | NA | NA | NA | NA | 84(5.4) | 84(5.4) |
| STEMA | 314(12.5) | 314(12.4) | NA | NA | NA | NA | NA | NA |
| Target lesion site, n (%) | | | | | | | | |
| Left main | 90(2.6) | 112(3.1) | NA | NA | 18(3) | 23(4) | 24(1.2) | 30(1.5) |
| Left anterior des-cending artery | 1768(50.6) | 1781(49.5) | 402(43) | 423(44) | 342(54) | 325(52) | 986(47.9) | 960(46.6) |
| Left circumflex artery | 651(18.6) | 715(19.9) | 133(14.3) | 137(14.2) | 209(33) | 181(29) | 481(23.4) | 501(24.3) |
| Right coronary artery | 972(27.8) | 976(27.1) | 206(22) | 207(21.6) | 191(30) | 222(36) | 567(27.6) | 571(27.7) |

| Characteristics | PRODIGY | | RESET | | EXCELLENT | | REAL-ZEST LATE | |
|---|---|---|---|---|---|---|---|---|
| | L-DAPT | DAPT 6Mo | DAPT 3Mo+ASA | DAPT 12Mo | DAPT 6Mo | DAPT 12Mo | L-DAPT | DAPT 12Mo |
| No. of participants | 987 | 983 | 1059 | 1058 | 722 | 721 | 1357 | 1344 |
| Age, mean(SD), years | 67.8±11 | 67.9±11 | 62.4±9.4 | 62.4±9.8 | 63.0±9.6 | 62.4±10.4 | 62.0±9.8 | 61.9±9.9 |
| Male, n/ total (%) | 764(77.4) | 747(76.0) | 682(64.4) | 665(62.9) | 470(65.1) | 461(63.9) | 950(70.0) | 933(69.4) |
| BMI, mean(SD), $kg/m^2$ | 26.6 | 26.7 | 25.0±3.2 | 24.9±3.1 | 24.9±3.1 | 25.1±3.0 | NA | NA |
| Cardiovascular risk factors, n/total (%) | | | | | | | | |
| Hypertension | 721(73.0) | 693(70.4) | 660(62.3) | 650(61.4) | 525(72.7) | 532(73.8) | 775(57.1) | 765(56.9) |
| Diabetes | 244(24.7) | 233(23.7) | 316(29.8) | 305(28.8) | 272(37.7) | 278(38.6) | 340(25.1) | 364(27.1) |
| Dyslipidemia | 553(56.0) | 525(53.4) | 611(57.7) | 634(59.9) | 543(75.2) | 550(76.3) | 586(43.2) | 584(43.5) |
| Current smoker | 222(22.5) | 247(25.1) | 267(25.2) | 241(22.8) | 198(27.4) | 186(25.8) | 404(29.8) | 431(32.1) |
| History, n/ total (%) | | | | | | | | |
| MI | 270(27.3) | 258(26.2) | 19(1.8) | 17(1.6) | 47(6.5) | 27(3.7) | 51(3.8) | 45(3.3) |
| PCI | 184(18.6) | 174(17.7) | 37(3.5) | 32(3.0) | 67(9.3) | 62(8.6) | 177(13.0) | 159(11.8) |
| Coronary artery bypass graft | 110(11.1) | 105(10.7) | 2(0.2) | 6(0.6) | 11(1.5) | 7(1.0) | NA | NA |
| Stroke | 37(3.7) | 39(4.0) | NA | NA | NA | NA | 57(4.2) | 45(3.3) |
| Clinical presenta-tion, n/ total (%) | | | | | | | | |
| Stable angina | 257(26.0) | 250(25.4) | 471(44.5) | 490(46.3) | 353(48.9) | 346(48.0) | 514(37.9) | 514(37.9) |
| UA | 183(18.5) | 182(18.5) | 432(40.8) | 422(39.9) | NA | NA | 514(37.9) | 514(37.9) |
| NSTEMA | 226(22.9) | 224(22.8) | NA | NA | NA | NA | 514(37.9) | 144(10.7) |
| STEMA | 321(32.5) | 327(33.3) | NA | NA | 19(2.6) | 26(3.6) | 155(11.4) | 141(10.5) |
| Target lesion site, n (%) | | | | | | | | |
| Left main | 55(5.6) | 56(5.7) | NA | NA | NA | NA | 55(2.9) | 44(2.4) |
| Left anterior descending artery | 518(52.5) | 518 (52.7) | 707(52.7) | 722(53.6) | 452(63.0) | 447(62.2) | 912(48.7) | 921(49.9) |
| Left circumflex artery | 321(32.5) | 318(32.4) | 281(21.0) | 259(19.2) | NA | NA | 921(49.9) | 334(18.1) |
| Right coronary artery | 346(35.1) | 363(36.9) | 353(26.3) | 365(27.1) | NA | NA | 334(18.1) | 546(29.6) |

NA, the concrete information was not available; PCI, percutaneous coronary intervention; MI, myocardial infarction; ST, stent thrombosis; DAPT, dual antiplatelet therapy; L-DAPT, longer than 12 months DAPT; DAPT 12Mo,12 months DAPT; S-DAPT+ASA, shorter than 6-month DAPT followed by aspirin monotherapy; S-DAPT+P2Y12, shorter than 6-month DAPT followed by a P2Y12 receptor inhibitor monotherapy; BMI, body mass index (calculated as the weight in kilograms divided by height in meters); NSTEMI, non-ST elevation myocardial infarction; STEMI, ST elevation myocardial infarction; UA, unstable angina; SD, standard deviation

## Supplementary Table S5 Definitions of clinical endpoints of the included trials

| Trials | Cardiac death | MI | Definite or probable ST | Major Bleeding | Any Bleeding | NACE |
|---|---|---|---|---|---|---|
| **TICO** | Death due to MI, cardiac perforation or pericardial tamponade, an arrhythmia or conduction abnormality, stroke within 30 days of the procedure or related to the procedure, death due to a procedural. | Symptoms, electrocardiographic changes, or abnormal imaging findings, combined with a CK-MB fraction above the upper normal limits or a troponin T or troponin I level greater than the 99th percentile of the upper normal limit. | ARC criteria | TIMI major bleeding | TIMI minor or major | The composite of major bleeding and major adverse cardiac and cerebrovascular events. |
| **STOPDAPT-2** | All deaths are considered cardiovascular deaths unless a definite non-cardiovascular cause can be established. | ARC criteria | ARC criteria | TIMI major bleeding | TIMI major or minor bleeding | A composite of cardiovascular and bleeding events (cardiovascular death, MI, definite ST, ischemic or hemorrhagic stroke, or TIMI major or minor bleeding). |
| **TWILIGHT** | Any death due to proximate cardiac cause, unwitnessed death and death of unknown cause, all procedure-related deaths including those related to concomitant treatment. | Third universal definition | ARC criteria | BARC criteria type 3 or 5 | BARC criteria type 2, 3 or 5 | NA |

| | | | | | | |
|---|---|---|---|---|---|---|
| **SMART-CHOICE** | All deaths were considered cardiac unless a definite non-cardiac cause could be established. | Elevated cardiac enzyme levels above the upper reference limit with ischemic symptoms or electrocardiographic findings indicative of ischemia. | ARC criteria | BARC criteria type 3,4 or 5 bleeding | BARC criteria type 2 to 5 bleeding | Major adverse cardiac and cerebrovascular events plus BARC type 2 to 5 bleeding. |
| **REDUCE** | All deaths are considered cardiovascular deaths unless a definite non-cardiovascular cause can be established. | Third universal definition of MI | ARC Criteria | NA | BARC criteria type 2, 3 or 5 | Composite of all-cause mortality, MI, ST, stroke, TVR or bleeding (BARC 2, 3, 5). |
| **GLOBAL LEADERS** | All deaths were considered cardiac unless a definite non-cardiac cause could be established. | Third universal MI definition | ARC criteria | BARC criteria type 3 or 5 | BARC criteria type 2, 3 or 5 | A composite of all-cause mortality, any stroke, MI, revascularization, BARC type 3 or 5 bleeding. |
| **OPTIMA-C** | ARC criteria | ARC criteria | ARC criteria | TIMI major bleeding | NA | NA |
| **DAPT-STEMI** | All deaths are considered cardiac unless an unequivocal non-cardiac cause can be established. | ARC criteria | ARC criteria | TIMI major bleeding | TIMI criteria | All-cause mortality, any MI, any revascularization, stroke, and TIMI major bleeding. |

| | | | | | | |
|---|---|---|---|---|---|---|
| **SMART-DATE** | All deaths were considered cardiac unless a definite non-cardiac cause could be established. | Elevated cardiac enzymes above the upper reference limit with ischemic symptoms or electrocardiography findings indicative of ischemia that was not related to the index procedure. | ARC criteria | BARC type 3-5 bleeding | BARC type 2-5 bleeding | Major adverse cardiac and cerebrovascular events plus BARC type 2–5 bleeding. |
| **NIPPON** | All deaths are considered to be cardiac deaths unless an unequivocal non-cardiac cause can be established. | (1)Per-procedural: A serum CK-MB level exceeding the ULN should not be considered as new MI, but as MI at registration; a serum troponin or serum CK-MB level exceeding 3 times the ULN within 48 hours after PCI; a serum troponin or serum CK-MB level exceeding 5 times the ULN within 72 hours after CABG; and a new Q-wave, left bundle block, new occlusion of the native vessel or graft, or reduction of viable myocardium on diagnostic imaging.(2)Spontaneous: When myocardial enzymes are at or above the ULN, it should be considered as MI at registration, and when the serum level of troponin or CK-MB exceeds ULN more than 48 hours after PCI or within 72 hours after CABG.(3)Re-infarction: Blood levels of biomarkers measured twice after the onset of MI are stable or decrease and the values at 3 to 6 hours after PCI show a > 20% increase compared with those obtained at index PCI. | ARC criteria | BARC 3 and 5 type bleeding | NA | Composite event of all-cause death, nonfatal MI, nonfatal stroke, or major bleeding. |
| **ITALIC** | All deaths unless an unequivocal non-cardiac cause could be established. | Q-wave MI was defined by recurrence of symptoms and/or development of new pathological Q waves in 2 | ARC criteria | TIMI major bleeding | TIMI Major or Minor | A composite of all-cause mortality, MI, stroke, TVR, |

| | | | | | | |
|---|---|---|---|---|---|---|
| | | or more contiguous leads with elevated CK, CK-MB, or troponin levels. Non–Q-wave MI was defined by >2-fold CK elevation with elevated CK-MB or troponin without new pathological Q waves. | | | Bleeding | major bleeding. |
| **IVUS-XPL** | All deaths were considered cardiac deaths unless a definite noncardiac cause could be established. | The presence of consistent clinical symptoms, electrocardiographic changes, or abnormal imaging findings, combined with a CK-MB fraction increase greater than the upper normal limit or an increase in troponin T or troponin I to >99th percentile of the upper normal limit. | ARC criteria | TIMI major bleeding | NA | Composite of cardiac death, MI, stroke, or TIMI major bleeding. |
| **I-LOVE-IT 2** | Any death due to an evident cardiac cause, any death related to PCI, unwitnessed death or death of unknown causes. | Typical rise and fall of troponin or CK-MB fraction with at least one of the following: ischemic symptoms, development of pathological Q waves, ischemic electrocardiographic changes, or pathological findings of an acute MI. | ARC criteria | BARC type≥3 bleeding | BARC criteria | A composite of all-cause death, all MI, stroke, and major bleeding (BARC type ≥3 bleeding). |
| **OPTIDUAL** | NA | The presence of clinical or electrocardiographic changes consistent with myocardial ischemia in the setting of increased cardiac biomarkers above the upper incorporates confounders to account for potential imbalances due to limit of normal in accordance with the universal definition. | ARC criteria | TIMI major bleeding | TIMI criteria | Composite of death, MI, stroke, and major bleeding. |
| **ISAR-SAFE** | NA | TIMI Criteria | ARC criteria | BARC criteria type 3 or 5 | BARC criteria type 2, 3 or 5 | The composite of death, MI, definite, or probable ST, stroke or TIMI major bleeding. |

| | | | | | | |
|---|---|---|---|---|---|---|
| **DAPT Study** | Any death due to immediate cardiac cause. Unwitnessed death and death of unknown cause will be classified as cardiac death. | The categories include per-procedual PCI, per-procedual CABG, spontaneous, silent, sudden death, and reinfarction, more details were provided in the online appendix of the original article. | ARC criteria | GUSTO severe bleeding | BARC criteria | NA |
| **DES LATE** | All deaths were considered to have resulted from cardiac causes unless an unequivocal non-cardiac cause could be established. | Universal definition of myocardial infarction | ARC criteria | TIMI major bleeding | NA | A composite of cardiac death, MI, stroke, ST, or TIMI major bleeding. |
| **SECURITY** | Any death without a non-cardiac cause. | Cardiac enzyme elevation above the upper normal limit associated with at least one ischemic symptom; development of Q waves on the electrocardiogram; electrocardiogram changes indicative of ischemia or coronary artery intervention | ARC criteria | BARC 3 or 5 type bleeding | BARC criteria | A composite of cardiac death, MI, stroke, definite or probable ST, or BARC type 2,3 or 5 bleeding. |
| **ARCTIC-Interruption** | NA | In patients with elevated biomarkers before PCI, positive diagnosis of reinfarction is made when all of the following criteria are present: (1) documentation that troponin level was decreasing; (2) troponin measured 6 hours after PCI is N3×ULN; (3) peak troponin level measured within 24 hours after the event is elevated by at least 50% above the previous level. In patients in whom biomarkers are normal or have returned to normal prior to PCI, periprocedural MI is defined when troponin measured 6 hours after PCI is N3×ULN. Measurements of biomarkers are requested before and 6 hours after PCI and at discharge. | ARC criteria | STEEPLE major bleeding | STEEPLE criteria | Any death, MI, ST, stroke or TIA, urgent revascularisation, TIMI major bleeding. |

| | | | | | | |
|---|---|---|---|---|---|---|
| **OPTIMIZE** | Any unknown causes of death or death that cannot be clearly attributed to a non-cardiac cause will be considered cardiac. | Classified as Q wave or non-Q wave; Periprocedural—within 48 hours post-PCI with baseline biomarker <ULN, rise in CKMB or troponin >3 times ULN; Spontaneous—CK-MB or troponin >ULN; Re-infarction—stable or decreasing biomarker values on 2 samples and >20% increase 3 to 6 hours post-intervention as compared to baseline samples. | ARC criteria | Intracranial, intraocular, or retroperitoneal hemorrhage; Clinically overt blood loss resulting in a decree-se in hemoglobin of more than 3g/d; Any decrease in hemoglobin of more than 4 g/dL; Transfusion of 1 or more units of packed red blood cells or whole blood; Bleeding causes hemodynamic compromise and requires intervention. | Major bleeding plus bleeding events that did not meet criteria for either major or severe or life-threatening bleeding. | Composite of death from all causes, MI, stroke, or major bleeding. |
| **PRODIGY** | All deaths were considered to be of cardiovascular causes unless an unequivocal noncardiovascular cause | Universal definition of myocardial infarction | ARC criteria | BARC type 3-5 bleeding | BARC type 2-5 bleeding | NA |

| | | | | | | |
|---|---|---|---|---|---|---|
| | could be established. | | | | | |
| **RESET** | All deaths are considered cardiovascular deaths unless a definite non-cardiovascular cause can be established. | Presence of clinical symptoms, electrocardiographic change or abnormal imaging findings of myocardial infarction combined with an increase in creatine kinase myocardial band fraction to greater than three times the upper limit of the normal range or troponin-T/troponin-I more than the 99th percentile of the upper normal limit, unrelated to an interventional procedure. | ARC criteria | TIMI major bleeding | TIMI criteria | A composite of cardiovascular death, MI, ST, TVR, or bleeding. |
| **EXCELLENT** | All deaths were considered cardiac unless a definite non-cardiac cause could be established. | During the first 48 hours after PCI, defined as an increase of cardiac enzyme 3 times above the upper limit of normal in stable patients; In patients with elevated baseline levels of cardiac enzyme, myocardial infarction was defined as a subsequent increase of >2-fold from baseline values; After the first 48hours, myocardial infarction was defined as the presence of clinical signs of MI combined with a CK-MB fraction or troponin T/troponin I increase higher than upper limit of normal. | ARC criteria | TIMI major bleeding | TIMI criteria | A composite of death, MI,stroke, ST, or TIMI major bleeding. |
| **REAL-ZEST LATE** | All deaths were considered cardiac unless a definite non-cardiac cause could be established. | Universal definition of myocardial infarction | ARC criteria | TIMI major bleeding | NA | NA |

NACE, net adverse clinical events; NA, the concrete definition was not available; MI, myocardial infarction; ARC, Academic Research Consortium; TIMI, thrombolysis in myocardial Infarction; BARC, Bleeding Academic Research Consortium; CK-MB, creatine kinase-myocardial band; GUSTO, The Global Use of Strategies to Open Occluded Arteries; STEEPLE, Enoxaparin versus Unfractionated Heparin in Elective Percutaneous Coronary Intervention; REPLACE, Randomized Evaluation of PCI Linking Angiomax to Reduced Clinical Events; ISTH, International Society on Thrombosis and Haemostasi

## Supplementary Table S6 The number of participants who had each outcome in each study

| Trial | Treatment | Total | All-cause mortality | Cardiac death | MI | Major bleeding | Any bleeding | Definite or probable ST | NACE |
|---|---|---|---|---|---|---|---|---|---|
| TICO | DAPT 3Mo+P2Y12 | 1527 | 16 | 7 | 6 | 25 | 53 | 6 | 59 |
| | DAPT 12Mo | 1529 | 23 | 12 | 11 | 45 | 83 | 4 | 89 |
| STOPDAPT-2 | DAPT 1Mo+P2Y12 | 1500 | 21 | 9 | 13 | 3 | 6 | 4 | 35 |
| | DAPT 12Mo | 1509 | 18 | 11 | 11 | 16 | 23 | 1 | 55 |
| TWILIGHT | DAPT 3Mo+P2Y12 | 3555 | 34 | 26 | 95 | 34 | 141 | 14 | NA |
| | DAPT 12Mo | 3564 | 45 | 37 | 95 | 69 | 250 | 19 | NA |
| SMART-CHOICE | DAPT 3Mo+P2Y12 | 1495 | 21 | 11 | 11 | 12 | 28 | 3 | 65 |
| | DAPT 12Mo | 1498 | 18 | 13 | 17 | 14 | 49 | 2 | 81 |
| REDUCE | DAPT 3Mo+ASA | 733 | 23 | 13 | 26 | 24 | 24 | 12 | 85 |
| | DAPT 12Mo | 727 | 16 | 8 | 22 | 29 | 29 | 6 | 88 |
| GLOBAL LEADERS | DAPT 1Mo+P2Y12 | 7980 | 224 | NA | 248 | 163 | 529 | 64 | 616 |
| | DAPT 12Mo | 7988 | 253 | NA | 250 | 169 | 532 | 64 | 653 |
| OPTIMA-C | DAPT 6Mo | 683 | 2 | 1 | 1 | 1 | 1 | 0 | NA |
| | DAPT 12Mo | 684 | 3 | 2 | 1 | 1 | 1 | 1 | NA |
| DAPT-STEMI | DAPT 6Mo | 433 | 3 | 2 | 8 | 1 | 3 | 3 | 21 |
| | DAPT 12Mo | 437 | 6 | 4 | 8 | 2 | 5 | 4 | 29 |
| SMART-DATE | DAPT 6Mo | 1357 | 35 | 18 | 24 | 6 | 35 | 15 | 96 |
| | L-DAPT | 1355 | 39 | 24 | 10 | 10 | 51 | 10 | 99 |

| | | | | | | | | | |
|---|---|---|---|---|---|---|---|---|---|
| NIPPON | L-DAPT | 1653 | 7 | 4 | 1 | 12 | 12 | 1 | 24 |
| | DAPT 6Mo | 1654 | 16 | 8 | 4 | 11 | 11 | 2 | 34 |
| IVUS-XPL | DAPT 6Mo | 699 | 5 | 3 | 1 | 5 | 5 | 2 | 15 |
| | DAPT 12Mo | 701 | 10 | 5 | 0 | 7 | 7 | 2 | 14 |
| OPTIDUAL | L-DAPT | 695 | 16 | 10 | 11 | 4 | 18 | 3 | 40 |
| | DAPT 12Mo | 690 | 24 | 14 | 16 | 4 | 20 | 1 | 52 |
| I-LOVE-IT 2 | DAPT 6Mo | 909 | 14 | 7 | 42 | 13 | 57 | 5 | 72 |
| | DAPT 12Mo | 920 | 17 | 8 | 37 | 7 | 60 | 2 | 67 |
| ISAR-SAFE | DAPT 6Mo | 1997 | 8 | NA | 13 | 6 | 21 | 5 | 29 |
| | DAPT 12Mo | 2003 | 12 | NA | 14 | 22 | 46 | 4 | 32 |
| ITALIC | DAPT 6Mo | 926 | 11 | 5 | 12 | 0 | 6 | 6 | 32 |
| | L-DAPT | 924 | 20 | 5 | 9 | 4 | 10 | 3 | 34 |
| DAPT Study | L-DAPT | 5020 | 98 | 45 | 99 | 119 | 263 | 19 | NA |
| | DAPT 12Mo | 4941 | 74 | 47 | 198 | 73 | 137 | 65 | NA |
| DES LATE | DAPT 12Mo | 2514 | 32 | 19 | 27 | 24 | 24 | 11 | 74 |
| | L-DAPT | 2531 | 46 | 28 | 19 | 34 | 34 | 7 | 89 |
| SECURITY | DAPT 6Mo | 682 | 14 | 6 | 21 | 5 | 6 | 3 | 46 |
| | DAPT 12Mo | 717 | 15 | 6 | 19 | 8 | 10 | 3 | 45 |
| ARCTIC-Interruption | L-DAPT | 635 | 7 | NA | 9 | 7 | 12 | 0 | 30 |
| | DAPT 12Mo | 624 | 9 | NA | 9 | 1 | 3 | 3 | 28 |
| OPTIMIZE | DAPT 3Mo+ASA | 1563 | 43 | 29 | 49 | 10 | 35 | 13 | 93 |
| | DAPT 12Mo | 1556 | 45 | 32 | 42 | 14 | 45 | 12 | 90 |
| PRODIGY | L-DAPT | 987 | 65 | 36 | 39 | 34 | 73 | 13 | NA |
| | DAPT 6Mo | 983 | 65 | 37 | 41 | 21 | 35 | 15 | NA |
| RESET | DAPT 3Mo+ASA | 1059 | 5 | 2 | 2 | 2 | 5 | 2 | 40 |

| | | | | | | | | | |
|---|---|---|---|---|---|---|---|---|---|
| | DAPT 12Mo | 1058 | 8 | 4 | 4 | 6 | 10 | 3 | 41 |
| EXCELLENT | DAPT 6Mo | 722 | 4 | 2 | 13 | 2 | 4 | 6 | 24 |
| | DAPT 12Mo | 721 | 7 | 3 | 7 | 4 | 10 | 1 | 21 |
| REAL-ZEST | L-DAPT | 1357 | 20 | 13 | 10 | 3 | 3 | 5 | NA |
| LATE | DAPT 12Mo | 1344 | 13 | 8 | 7 | 1 | 1 | 4 | NA |

NA, the concrete outcomes was not available; MI, myocardial infarction; ST, stent thrombosis; ANCE, net adverse clinical events; DAPT, dual antiplatelet therapy; L-DAPT, longer than 12 months DAPT; DAPT 12Mo,12 months DAPT; DAPT 6Mo,6 months DAPT; DAPT 3Mo+ASA, 3-months DAPT followed by aspirin monotherapy; DAPT 3Mo+P2Y12, 3-month DAPT followed by a P2Y12 receptor inhibitor monotherapy; DAPT 1Mo+P2Y12,1-month DAPT followed by a P2Y12 receptor inhibitor monotherapy.

## Supplementary Table S7 Risk of bias of included trials using the Cochrane Risk Assessment Tool

| Trial | Random sequence generation (selection bias) | Allocation concealment (selection bias) | Blinding of participants and personnel* (performance bias) | Blinding of outcome assessment (detection bias) | Incomplete outcome data** (attrition bias) | Selective reporting (reporting bias) | Other sources of bias |
|---|---|---|---|---|---|---|---|
| TICO | + | + | ? | + | + | + | ? |
| STOPDAPT-2 | + | ? | ? | + | + | + | ? |
| TWILIGHT | + | + | + | + | + | + | ? |
| SMART-CHOICE | + | + | ? | + | + | + | ? |
| REDUCE | + | + | - | ? | + | + | - |
| GLOBAL LEADERS | + | + | + | ? | + | + | ? |
| OPTIMA-C | + | + | + | + | + | + | ? |
| DAPT-STEMI | + | ? | ? | + | + | + | ? |
| SMART-DATE | + | + | + | ? | + | + | ? |
| NIPPON | + | + | ? | + | + | + | ? |
| ITALIC | + | ? | ? | + | + | + | ? |
| IVUS-XPL | + | + | ? | + | + | + | - |
| I-LOVE-IT 2 | + | ? | ? | ? | + | + | ? |
| OPTIDUAL | + | + | + | + | + | + | ? |
| ISAR-SAFE | + | + | + | + | + | + | ? |
| DAPT Study | ? | ? | + | - | + | + | ? |

| | | | | | | | |
|---|---|---|---|---|---|---|---|
| DES LATE | + | + | - | - | + | + | - |
| SECURITY | + | + | ? | ? | ? | + | - |
| ARCTIC-Interruption | + | + | + | ? | + | + | ? |
| OPTIMIZE | + | + | + | + | + | + | ? |
| PRODIGY | + | + | ? | ? | + | + | ? |
| RESET | + | + | ? | ? | + | + | - |
| EXCELLENT | + | + | ? | ? | + | + | - |
| REAL-ZEST LATE | + | + | - | - | + | + | - |

*Despite that the trials used an open label trial design, the outcome assessment was blinded, and therefore the study design did not influence reported outcomes and hereby negatively effecting the quality of evidence.

**All trials reported minimal loss to follow-up.

Green refers to a low risk of bias; yellow refers to the uncertain risk of bias; red refers to a high risk of bias.

**Supplementary Table S8 Pooled estimates of the network meta-analysis of each endpoint.** Results in the upper triangle are odds ratios (95% confidence intervals) from the network meta-analysis between the column defining intervention and row defining intervention. Significant results are in bold.

**All-cause mortality**

| | | | | | |
|---|---|---|---|---|---|
| DAPT 12Mo | 0.93 (0.66, 1.40) | 1.00 (0.66, 1.55) | 0.84 (0.57, 1.25) | 0.89 (0.64, 1.21) | 1.06 (0.79, 1.35) |
| 1.07 (0.72, 1.52) | DAPT 1Mo+P2Y12 | 1.08 (0.60, 1.88) | 0.89 (0.51, 1.52) | 0.96 (0.55, 1.48) | 1.13 (0.67, 1.71) |
| 1.00 (0.64, 1.51) | 0.93 (0.53, 1.66) | DAPT 3Mo+ASA | 0.84 (0.46, 1.46) | 0.89 (0.50, 1.48) | 1.06 (0.62, 1.70) |
| 1.19 (0.80, 1.77) | 1.12 (0.66, 1.96) | 1.20 (0.68, 2.18) | DAPT 3Mo+P2Y12 | 1.07 (0.62, 1.70) | 1.27 (0.76, 2.01) |
| 1.12 (0.83, 1.57) | 1.04 (0.68, 1.82) | 1.12 (0.68, 1.99) | 0.94 (0.59, 1.60) | DAPT 6Mo | 1.18 (0.91, 1.59) |
| 0.95 (0.74, 1.27) | 0.88 (0.59, 1.49) | 0.94 (0.59, 1.62) | 0.79 (0.50, 1.31) | 0.85 (0.63, 1.10) | L-DAPT |

**Cardiac death**

| | | | | | |
|---|---|---|---|---|---|
| DAPT 12Mo | 0.78 (0.31, 1.99) | 0.99 (0.61, 1.63) | 0.70 (0.45, 1.09) | 0.93 (0.63, 1.39) | 1.02 (0.73, 1.40) |
| 1.28 (0.50, 3.23) | DAPT 1Mo+P2Y12 | 1.26 (0.45, 3.55) | 0.90 (0.32, 2.58) | 1.19 (0.42, 3.26) | 1.32 (0.48, 3.45) |
| 1.01 (0.61, 1.64) | 0.80 (0.28, 2.20) | DAPT 3Mo+ASA | 0.73 (0.35, 1.37) | 0.97 (0.49, 1.76) | 1.04 (0.57, 1.87) |
| 1.43 (0.92, 2.23) | 1.12 (0.39, 3.09) | 1.37 (0.73, 2.87) | DAPT 3Mo+P2Y12 | 1.31 (0.73, 2.37) | 1.45 (0.84, 2.46) |
| 1.08 (0.72, 1.59) | 0.84 (0.31, 2.39) | 1.03 (0.57, 2.03) | 0.76 (0.42, 1.38) | DAPT 6Mo | 1.11 (0.78, 1.54) |
| 0.98 (0.71, 1.36) | 0.76 (0.29, 2.10) | 0.96 (0.53, 1.76) | 0.69 (0.41, 1.19) | 0.90 (0.65, 1.29) | L-DAPT |

**Myocardial infarction**

| | | | | | |
|---|---|---|---|---|---|
| DAPT 12Mo | 1.03 (0.66, 1.70) | 1.10 (0.69, 1.75) | 0.84 (0.52, 1.27) | 1.12 (0.82, 1.57) | **0.68 (0.51, 0.95)** |
| 0.97 (0.59, 1.51) | DAPT 1Mo+P2Y12 | 1.07 (0.53, 2.04) | 0.82 (0.40, 1.45) | 1.08 (0.63, 1.92) | 0.66 (0.38, 1.17) |
| 0.91 (0.57, 1.46) | 0.93 (0.49, 1.90) | DAPT 3Mo+ASA | 0.76 (0.39, 1.41) | 1.01 (0.59, 1.84) | 0.62 (0.36, 1.12) |
| 1.19 (0.79, 1.93) | 1.22 (0.69, 2.51) | 1.32 (0.71, 2.56) | DAPT 3Mo+P2Y12 | 1.33 (0.80, 2.45) | 0.81 (0.50, 1.51) |

| | | | | | |
|---|---|---|---|---|---|
| 0.89 (0.64, 1.22) | 0.93 (0.52, 1.60) | 0.99 (0.54, 1.71) | 0.75 (0.41, 1.25) | DAPT 6Mo | **0.61 (0.43, 0.87)** |
| **1.47 (1.06, 1.95)** | 1.52 (0.86, 2.62) | 1.62 (0.89, 2.78) | 1.24 (0.66, 2.02) | **1.63 (1.15, 2.34)** | L-DAPT |
| **Major bleeding** | | | | | |
| DAPT 12Mo | 0.67 (0.26, 1.23) | 0.67 (0.31, 1.31) | 0.58 (0.32, 1.07) | 0.77 (0.46, 1.23) | 1.50 (0.95, 2.54) |
| 1.49 (0.81, 3.92) | DAPT 1Mo+P2Y12 | 1.01 (0.39, 3.15) | 0.86 (0.37, 2.81) | 1.14 (0.52, 3.39) | **2.22 (1.07, 7.12)** |
| 1.49 (0.76, 3.27) | 0.99 (0.32, 2.59) | DAPT 3Mo+ASA | 0.85 (0.36, 2.37) | 1.15 (0.50, 2.86) | **2.26 (1.02, 5.82)** |
| 1.74 (0.94, 3.11) | 1.16 (0.36, 2.68) | 1.18 (0.42, 2.77) | DAPT 3Mo+P2Y12 | 1.33 (0.60, 2.75) | **2.62 (1.21, 5.63)** |
| 1.31 (0.81, 2.17) | 0.87 (0.29, 1.91) | 0.87 (0.35, 2.01) | 0.75 (0.36, 1.67) | DAPT 6Mo | **1.98 (1.21, 3.50)** |
| 0.67 (0.39, 1.05) | **0.45 (0.14, 0.93)** | **0.44 (0.17, 0.98)** | **0.38 (0.18, 0.82)** | **0.50 (0.29, 0.83)** | L-DAPT |
| **Any bleeding** | | | | | |
| DAPT 12Mo | 0.74 (0.38, 1.18) | 0.74 (0.43, 1.22) | **0.58 (0.38, 0.88)** | 0.76 (0.52, 1.04) | **1.48 (1.02, 2.03)** |
| 1.35 (0.85, 2.65) | DAPT 1Mo+P2Y12 | 1.00 (0.51, 2.29) | 0.77 (0.43, 1.77) | 1.02 (0.58, 2.11) | **1.99 (1.13, 4.14)** |
| 1.35 (0.82, 2.30) | 1.00 (0.44, 1.95) | DAPT 3Mo+ASA | 0.79 (0.41, 1.54) | 1.03 (0.56, 1.88) | **2.01 (1.07, 3.67)** |
| **1.73 (1.14, 2.63)** | 1.29 (0.57, 2.32) | 1.27 (0.65, 2.44) | DAPT 3Mo+P2Y12 | 1.31 (0.75, 2.20) | **2.56 (1.46, 4.29)** |
| 1.32 (0.96, 1.91) | 0.98 (0.47, 1.71) | 0.97 (0.53, 1.80) | 0.76 (0.45, 1.34) | DAPT 6Mo | **1.95 (1.37, 2.74)** |
| **0.68 (0.49, 0.98)** | **0.50 (0.24, 0.89)** | **0.50 (0.27, 0.94)** | **0.39 (0.23, 0.68)** | **0.51 (0.37, 0.73)** | L-DAPT |
| **Definite or probable stent thrombosis** | | | | | |
| DAPT 12Mo | 1.20 (0.54, 3.65) | 1.25 (0.54, 2.95) | 1.00 (0.44, 2.46) | 1.09 (0.62, 2.08) | 0.58 (0.34, 1.11) |
| 0.83 (0.27, 1.85) | DAPT 1Mo+P2Y12 | 1.03 (0.25, 3.28) | 0.82 (0.21, 2.75) | 0.89 (0.28, 2.53) | 0.47 (0.15, 1.35) |
| 0.80 (0.34, 1.87) | 0.97 (0.30, 3.96) | DAPT 3Mo+ASA | 0.79 (0.25, 2.80) | 0.87 (0.32, 2.55) | 0.46 (0.18, 1.38) |
| 1.00 (0.41, 2.29) | 1.23 (0.36, 4.73) | 1.26 (0.36, 4.05) | DAPT 3Mo+P2Y12 | 1.10 (0.39, 3.10) | 0.58 (0.21, 1.67) |
| 0.92 (0.48, 1.62) | 1.12 (0.39, 3.58) | 1.15 (0.39, 3.13) | 0.91 (0.32, 2.58) | DAPT 6Mo | **0.53 (0.29, 0.98)** |
| 1.73 (0.90, 2.91) | 2.11 (0.74, 6.59) | 2.16 (0.72, 5.66) | 1.72 (0.60, 4.73) | **1.88 (1.02, 3.40)** | L-DAPT |
| **Net adverse clinical events** | | | | | |
| DAPT 12Mo | 0.89 (0.68, 1.06) | 0.98 (0.78, 1.24) | **0.72 (0.53, 0.95)** | 1.02 (0.84, 1.22) | 1.00 (0.80, 1.23) |

| | | | | | |
|---|---|---|---|---|---|
| 1.12 (0.94, 1.47) | DAPT 1Mo+P2Y12 | 1.11 (0.82, 1.60) | 0.81 (0.58, 1.20) | 1.15 (0.89, 1.59) | 1.12 (0.86, 1.56) |
| 1.02 (0.81, 1.29) | 0.90 (0.63, 1.21) | DAPT 3Mo+ASA | 0.73 (0.51, 1.05) | 1.04 (0.77, 1.40) | 1.02 (0.74, 1.38) |
| **1.40 (1.05, 1.87)** | 1.23 (0.83, 1.74) | 1.37 (0.95, 1.96) | DAPT 3Mo+P2Y12 | **1.42 (1.01, 2.04)** | 1.40 (0.97, 1.95) |
| 0.98 (0.82, 1.19) | 0.87 (0.63, 1.13) | 0.96 (0.72, 1.31) | **0.70 (0.49, 0.99)** | DAPT 6Mo | 0.98 (0.80, 1.20) |
| 1.00 (0.82, 1.24) | 0.89 (0.64, 1.17) | 0.98 (0.72, 1.36) | 0.72 (0.51, 1.03) | 1.02 (0.83, 1.26) | L-DAPT |

**Supplementary Table S9 Node-splitting analysis of Inconsistency for all endpoints**

| Endpoints | Comparison | Direct effect | Indirect effect | Overall | P-value |
|---|---|---|---|---|---|
| All-cause mortality | DAPT 12Mo vs DAPT 6Mo | -0.33 (-0.74, 0.04) | 0.13 (-0.30, 0.57) | -0.11 (-0.45, 0.19) | 0.09 |
| | DAPT 12Mo vs L-DAPT | 0.17 (-0.14, 0.45) | -0.31 (-0.86, 0.20) | 0.05 (-0.24, 0.30) | 0.10 |
| | DAPT 6Mo vs L-DAPT | 0.02 (-0.28, 0.34) | 0.51 (0.02, 0.99) | 0.16 (-0.10, 0.46) | 0.12 |
| Cardiac death | DAPT 12Mo vs DAPT 6Mo | -0.29 (-0.91, 0.28) | 0.09 (-0.46, 0.65) | -0.08 (-0.47, 0.33) | 0.33 |
| | DAPT 12Mo vs L-DAPT | 0.08 (-0.27, 0.44) | -0.30 (-1.03, 0.42) | 0.02 (-0.31, 0.34) | 0.34 |
| | DAPT 6Mo vs L-DAPT | 0.00 (-0.40, 0.39) | 0.37 (-0.29, 1.07) | 0.10 (-0.25, 0.43) | 0.32 |
| Myocardial infarction | DAPT 12Mo vs DAPT 6Mo | 0.18 (-0.18, 0.54) | -0.08 (-0.63, 0.57) | 0.11 (-0.20, 0.45) | 0.42 |
| | DAPT 12Mo vs L-DAPT | -0.47 (-0.79, -0.05) | -0.19 (-0.79, 0.36) | -0.38 (-0.67, -0.06) | 0.44 |
| | DAPT 6Mo vs L-DAPT | -0.38 (-0.83, 0.04) | -0.65 (-1.10, -0.09) | -0.49 (-0.85, -0.14) | 0.41 |
| Major bleeding | DAPT 12Mo vs DAPT 6Mo | -0.41 (-1.00, 0.19) | 0.02 (-0.86, 0.92) | -0.27 (-0.77, 0.21) | 0.40 |
| | DAPT 12Mo vs L-DAPT | 0.52 (0.00, 1.16) | 0.10 (-0.76, 1.03) | 0.41 (-0.05, 0.93) | 0.39 |
| | DAPT 6Mo vs L-DAPT | 0.53 (-0.10, 1.25) | 0.95 (0.16, 1.85) | 0.68 (0.19, 1.25) | 0.40 |
| Any bleeding | DAPT 12Mo vs DAPT 6Mo | -0.44 (-0.87, -0.05) | 0.00 (-0.59, 0.63) | -0.28 (-0.65, 0.04) | 0.20 |
| | DAPT 12Mo vs L-DAPT | 0.52 (0.11, 0.93) | 0.07 (-0.57, 0.65) | 0.39 (0.02, 0.71) | 0.19 |
| | DAPT 6Mo vs L-DAPT | 0.51 (0.05, 0.94) | 0.96 (0.40, 1.56) | 0.67 (0.32, 1.01) | 0.19 |
| Definite or probable stent thrombosis | DAPT 12Mo vs DAPT 6Mo | 0.36 (-0.37, 1.07) | -0.41 (-1.23, 0.68) | 0.09 (-0.48, 0.73) | 0.23 |
| | DAPT 12Mo vs L-DAPT | -0.78 (-1.37, -0.04) | -0.02 (-1.04, 0.95) | -0.55 (-1.07, 0.11) | 0.25 |
| | DAPT 6Mo vs L-DAPT | -0.39 (-1.12, 0.29) | -1.12 (-2.07, -0.08) | -0.63 (-1.22, -0.02) | 0.25 |
| Net adverse clinical | DAPT 12Mo vs DAPT 6Mo | -0.00 (-0.22, 0.22) | 0.04 (-0.32, 0.44) | 0.02 (-0.17, 0.20) | 0.81 |

| | | | | | |
|---|---|---|---|---|---|
| events | DAPT 12Mo vs L-DAPT | 0.02 (-0.26, 0.27) | -0.04 (-0.40, 0.32) | -0.00 (-0.22, 0.20) | 0.80 |
| | DAPT 6Mo vs L-DAPT | -0.03 (-0.32, 0.23) | 0.01 (-0.36, 0.37) | -0.02 (-0.23, 0.18) | 0.87 |

DAPT, dual antiplatelet therapy; L-DAPT, longer than 12 months DAPT; DAPT 12Mo,12 months DAPT; DAPT 6Mo,6 months DAPT; DAPT 3Mo+ASA, 3-months DAPT followed by aspirin monotherapy; DAPT 3Mo+P2Y12, 3-month DAPT followed by a P2Y12 receptor inhibitor monotherapy; DAPT 1Mo+P2Y12,1-month DAPT followed by a P2Y12 receptor inhibitor monotherapy.

## Supplementary Table S10 Pooled estimates of the network meta-analysis of sensitivity analysis

**All-cause mortality**

| | | | | | |
|---|---|---|---|---|---|
| DAPT 12Mo | 0.93 (0.63, 1.49) | 1.03 (0.65, 1.61) | 0.82 (0.55, 1.26) | 0.87 (0.61, 1.19) | 1.05 (0.77, 1.37) |
| 1.08 (0.67, 1.58) | DAPT 1Mo+P2Y12 | 1.11 (0.57, 1.97) | 0.89 (0.48, 1.58) | 0.93 (0.50, 1.51) | 1.13 (0.63, 1.78) |
| 0.97 (0.62, 1.54) | 0.90 (0.51, 1.77) | DAPT 3Mo+ASA | 0.80 (0.44, 1.50) | 0.85 (0.47, 1.45) | 1.02 (0.59, 1.71) |
| 1.22 (0.79, 1.82) | 1.13 (0.63, 2.10) | 1.26 (0.67, 2.27) | DAPT 3Mo+P2Y12 | 1.05 (0.59, 1.73) | 1.28 (0.74, 2.07) |
| 1.15 (0.84, 1.64) | 1.07 (0.66, 1.99) | 1.18 (0.69, 2.11) | 0.95 (0.58, 1.68) | DAPT 6Mo | 1.21 (0.87, 1.69) |
| 0.95 (0.73, 1.30) | 0.88 (0.56, 1.60) | 0.98 (0.58, 1.70) | 0.78 (0.48, 1.34) | 0.83 (0.59, 1.15) | L-DAPT |

**Cardiac death**

| | | | | | |
|---|---|---|---|---|---|
| DAPT 12Mo | 0.78 (0.31, 2.13) | 1.00 (0.60, 1.68) | 0.72 (0.45, 1.10) | 0.97 (0.63, 1.44) | 1.00 (0.72, 1.39) |
| 1.28 (0.47, 3.19) | DAPT 1Mo+P2Y12 | 1.26 (0.41, 3.71) | 0.90 (0.29, 2.47) | 1.23 (0.42, 3.41) | 1.28 (0.45, 3.26) |
| 1.00 (0.60, 1.65) | 0.79 (0.27, 2.46) | DAPT 3Mo+ASA | 0.73 (0.36, 1.40) | 0.98 (0.51, 1.85) | 1.00 (0.55, 1.80) |
| 1.39 (0.91, 2.23) | 1.12 (0.40, 3.40) | 1.37 (0.71, 2.79) | DAPT 3Mo+P2Y12 | 1.33 (0.74, 2.49) | 1.40 (0.81, 2.52) |
| 1.03 (0.69, 1.58) | 0.81 (0.29, 2.40) | 1.02 (0.54, 1.96) | 0.75 (0.40, 1.35) | DAPT 6Mo | 1.03 (0.70, 1.53) |
| 1.00 (0.72, 1.39) | 0.78 (0.31, 2.22) | 1.00 (0.56, 1.82) | 0.71 (0.40, 1.24) | 0.97 (0.65, 1.44) | L-DAPT |

**Myocardial infarction**

| | | | | | |
|---|---|---|---|---|---|
| DAPT 12Mo | 1.03 (0.66, 1.63) | 1.11 (0.69, 1.75) | 0.84 (0.52, 1.27) | 1.06 (0.78, 1.49) | **0.69 (0.52, 0.99)** |
| 0.97 (0.61, 1.50) | DAPT 1Mo+P2Y12 | 1.08 (0.55, 2.01) | 0.82 (0.42, 1.45) | 1.02 (0.60, 1.83) | 0.67 (0.40, 1.20) |
| 0.90 (0.57, 1.46) | 0.92 (0.50, 1.80) | DAPT 3Mo+ASA | 0.77 (0.39, 1.43) | 0.95 (0.56, 1.73) | 0.62 (0.37, 1.14) |
| 1.18 (0.79, 1.93) | 1.22 (0.69, 2.38) | 1.30 (0.70, 2.54) | DAPT 3Mo+P2Y12 | 1.25 (0.75, 2.34) | 0.82 (0.49, 1.54) |
| 0.95 (0.67, 1.28) | 0.98 (0.55, 1.67) | 1.05 (0.58, 1.79) | 0.80 (0.43, 1.34) | DAPT 6Mo | **0.65 (0.46, 0.96)** |
| **1.45 (1.01, 1.93)** | 1.50 (0.83, 2.48) | 1.60 (0.88, 2.70) | 1.22 (0.65, 2.05) | **1.53 (1.04, 2.17)** | L-DAPT |

**Major bleeding**

| | | | | | |
|---|---|---|---|---|---|
| DAPT 12Mo | 0.65 (0.24, 1.29) | 0.67 (0.31, 1.37) | 0.58 (0.31, 1.13) | 0.76 (0.44, 1.25) | 1.53 (0.95, 2.75) |
| 1.54 (0.77, 4.24) | DAPT 1Mo+P2Y12 | 1.03 (0.37, 3.63) | 0.89 (0.36, 3.08) | 1.18 (0.49, 3.53) | **2.38 (1.05, 8.20)** |
| 1.50 (0.73, 3.26) | 0.97 (0.28, 2.67) | DAPT 3Mo+ASA | 0.86 (0.34, 2.41) | 1.13 (0.46, 2.90) | **2.30 (1.00, 6.28)** |
| 1.73 (0.89, 3.27) | 1.12 (0.32, 2.78) | 1.16 (0.41, 2.93) | DAPT 3Mo+P2Y12 | 1.31 (0.54, 2.88) | **2.66 (1.17, 6.36)** |
| 1.32 (0.80, 2.27) | 0.85 (0.28, 2.02) | 0.89 (0.35, 2.18) | 0.76 (0.35, 1.85) | DAPT 6Mo | **2.04 (1.17, 3.93)** |
| 0.65 (0.36, 1.05) | **0.42 (0.12, 0.95)** | **0.44 (0.16, 1.00)** | **0.38 (0.16, 0.85)** | **0.49 (0.25, 0.85)** | L-DAPT |

**Any bleeding**

| | | | | | |
|---|---|---|---|---|---|
| DAPT 12Mo | 0.75 (0.37, 1.16) | 0.73 (0.43, 1.21) | **0.58 (0.37, 0.87)** | **0.72 (0.50, 1.00)** | **1.53 (1.05, 2.12)** |
| 1.33 (0.87, 2.72) | DAPT 1Mo+P2Y12 | 0.98 (0.50, 2.40) | 0.76 (0.43, 1.79) | 0.97 (0.55, 2.10) | **2.02 (1.18, 4.33)** |
| 1.37 (0.82, 2.31) | 1.02 (0.42, 1.99) | DAPT 3Mo+ASA | 0.79 (0.41, 1.52) | 0.99 (0.53, 1.84) | **2.10 (1.09, 3.84)** |
| **1.74 (1.15, 2.70)** | 1.32 (0.56, 2.33) | 1.27 (0.66, 2.45) | DAPT 3Mo+P2Y12 | 1.26 (0.72, 2.16) | **2.67 (1.50, 4.52)** |
| **1.39 (1.00, 1.99)** | 1.04 (0.48, 1.80) | 1.01 (0.54, 1.90) | 0.79 (0.46, 1.39) | DAPT 6Mo | **2.10 (1.42, 3.10)** |
| **0.65 (0.47, 0.96)** | **0.50 (0.23, 0.85)** | **0.48 (0.26, 0.91)** | **0.37 (0.22, 0.67)** | **0.48 (0.32, 0.71)** | L-DAPT |

**Definite or probable stent thrombosis**

| | | | | | |
|---|---|---|---|---|---|
| DAPT 12Mo | 1.23 (0.52, 3.79) | 1.29 (0.52, 3.06) | 1.00 (0.43, 2.64) | 1.15 (0.62, 2.28) | 0.56 (0.32, 1.11) |
| 0.81 (0.26, 1.93) | DAPT 1Mo+P2Y12 | 1.04 (0.24, 3.40) | 0.80 (0.20, 2.94) | 0.91 (0.27, 2.86) | 0.45 (0.14, 1.38) |
| 0.78 (0.33, 1.91) | 0.96 (0.29, 4.21) | DAPT 3Mo+ASA | 0.79 (0.24, 2.99) | 0.90 (0.32, 2.81) | 0.45 (0.16, 1.37) |
| 1.00 (0.38, 2.34) | 1.26 (0.34, 4.92) | 1.27 (0.33, 4.13) | DAPT 3Mo+P2Y12 | 1.12 (0.37, 3.34) | 0.57 (0.19, 1.67) |
| 0.87 (0.44, 1.61) | 1.10 (0.35, 3.76) | 1.12 (0.36, 3.15) | 0.90 (0.30, 2.67) | DAPT 6Mo | 0.50 (0.25, 1.01) |
| 1.77 (0.90, 3.10) | 2.21 (0.72, 7.35) | 2.23 (0.73, 6.21) | 1.77 (0.60, 5.24) | 2.00 (0.99, 4.02) | L-DAPT |

**Net adverse clinical events**

| | | | | | |
|---|---|---|---|---|---|
| DAPT 12Mo | 0.88 (0.66, 1.07) | 1.00 (0.78, 1.25) | **0.71 (0.54, 0.96)** | 1.04 (0.85, 1.26) | 0.98 (0.77, 1.22) |
| 1.14 (0.93, 1.51) | DAPT 1Mo+P2Y12 | 1.14 (0.83, 1.64) | 0.81 (0.58, 1.25) | 1.19 (0.88, 1.66) | 1.12 (0.81, 1.56) |
| 1.00 (0.80, 1.28) | 0.88 (0.61, 1.21) | DAPT 3Mo+ASA | 0.72 (0.50, 1.06) | 1.05 (0.76, 1.42) | 0.98 (0.71, 1.36) |
| **1.40 (1.04, 1.85)** | 1.24 (0.80, 1.72) | 1.39 (0.94, 1.98) | DAPT 3Mo+P2Y12 | **1.46 (1.02, 2.02)** | 1.37 (0.93, 1.93) |

| | | | | | |
|---|---|---|---|---|---|
| 0.96 (0.79, 1.18) | 0.84 (0.60, 1.13) | 0.95 (0.71, 1.32) | **0.69 (0.50, 0.98)** | DAPT 6Mo | 0.93 (0.73, 1.22) |
| 1.02 (0.82, 1.31) | 0.89 (0.64, 1.23) | 1.02 (0.74, 1.40) | 0.73 (0.52, 1.07) | 1.07 (0.82, 1.38) | L-DAPT |

## Supplementary Table S11 The number of participants with acute coronary syndrome who had each outcome

| Trial | Treatment | Total | All-cause mortality | MI | Major bleeding | Any bleeding | Definite or probable ST | NACE |
|---|---|---|---|---|---|---|---|---|
| TICO | DAPT 3Mo+P2Y12 | 1527 | 16 | 6 | 25 | 53 | 6 | 59 |
| | DAPT 12Mo | 1529 | 23 | 11 | 45 | 83 | 4 | 89 |
| GLOBAL LEADERS | DAPT 1Mo+P2Y12 | 3750 | 38 | 50 | 28 | 97 | 7 | 248 |
| | DAPT 12Mo | 3737 | 51 | 52 | 54 | 152 | 6 | 266 |
| REDUCE | DAPT 3Mo+ASA | 733 | 23 | 26 | 24 | 24 | 12 | 85 |
| | DAPT 12Mo | 727 | 16 | 22 | 29 | 29 | 6 | 88 |
| DAPT-STEMI | DAPT 6Mo | 433 | 3 | 8 | 1 | 3 | 3 | 21 |
| | DAPT 12Mo | 437 | 6 | 8 | 2 | 5 | 4 | 29 |
| IVUS-XPL+EXCELLENT +RESET | DAPT 6Mo | 1119 | 9 | 7 | 4 | 9 | 6 | 22 |
| | DAPT 12Mo | 1097 | 11 | 5 | 6 | 14 | 4 | 21 |
| SMART-DATE | DAPT 6Mo | 1357 | 35 | 24 | 6 | 35 | 15 | 96 |
| | L-DAPT | 1355 | 39 | 10 | 10 | 51 | 10 | 99 |
| ITALIC | DAPT 6Mo | 400 | 4 | 7 | 0 | 2 | 5 | 14 |
| | L-DAPT | 406 | 9 | 6 | 2 | 6 | 3 | 17 |
| PRODIGY | L-DAPT | 733 | 56 | 39 | 17 | 30 | 15 | 103 |
| | DAPT 6Mo | 732 | 52 | 33 | 23 | 52 | 10 | 118 |
| ISAR-SAFE | DAPT 6Mo | 794 | 5 | 6 | 1 | 3 | 2 | 14 |
| | DAPT 12Mo | 807 | 7 | 8 | 2 | 5 | 2 | 17 |

| | | | | | | | | |
|---|---|---|---|---|---|---|---|---|
| DAPT Study | DAPT 12Mo | 1771 | 27 | 88 | 14 | 35 | 32 | NA |
| | L-DAPT | 1805 | 24 | 39 | 34 | 76 | 9 | NA |
| TWILIGHT | DAPT 3Mo+P2Y12 | 2273 | 22 | 70 | 17 | 81 | 8 | NA |
| | DAPT 12Mo | 2341 | 34 | 72 | 49 | 175 | 14 | NA |

NA, the concrete outcomes was not available; NACE, net adverse clinical events; MI, myocardial infarction; ST, stent thrombosis; DAPT, dual antiplatelet therapy; L-DAPT, longer than 12 months DAPT; DAPT 12Mo,12 months DAPT; DAPT 6Mo,6 months DAPT; DAPT 3Mo+ASA, 3-months DAPT followed by aspirin monotherapy; DAPT 3Mo+P2Y12, 3-month DAPT followed by a P2Y12 receptor inhibitor monotherapy; DAPT 1Mo+P2Y12,1-month DAPT followed by a P2Y12 receptor inhibitor monotherapy.

**Supplementary Table S12 Pooled estimates of the network meta-analysis of each endpoint in participants with acute coronary syndrome**

**All-cause mortality**

| | | | | | |
|---|---|---|---|---|---|
| DAPT 12Mo | 0.74 (0.39, 1.43) | 1.44 (0.63, 3.45) | 0.68 (0.39, 1.17) | 0.75 (0.43, 1.25) | 0.83 (0.48, 1.45) |
| 1.36 (0.70, 2.59) | DAPT 1Mo+P2Y12 | 1.96 (0.68, 5.58) | 0.92 (0.39, 2.16) | 1.02 (0.42, 2.33) | 1.14 (0.47, 2.63) |
| 0.69 (0.29, 1.59) | 0.51 (0.18, 1.47) | DAPT 3Mo+ASA | 0.46 (0.17, 1.23) | 0.52 (0.18, 1.35) | 0.57 (0.21, 1.56) |
| 1.47 (0.85, 2.55) | 1.09 (0.46, 2.59) | 2.16 (0.81, 6.02) | DAPT 3Mo+P2Y12 | 1.12 (0.50, 2.32) | 1.23 (0.56, 2.68) |
| 1.33 (0.80, 2.34) | 0.98 (0.43, 2.41) | 1.92 (0.74, 5.41) | 0.89 (0.43, 2.02) | DAPT 6Mo | 1.10 (0.77, 1.73) |
| 1.20 (0.69, 2.09) | 0.88 (0.38, 2.11) | 1.74 (0.64, 4.75) | 0.81 (0.37, 1.77) | 0.91 (0.58, 1.30) | L-DAPT |

**Myocardial infarction**

| | | | | | |
|---|---|---|---|---|---|
| DAPT 12Mo | 0.96 (0.41, 2.23) | 1.16 (0.45, 2.98) | 0.86 (0.40, 1.62) | 0.82 (0.46, 1.59) | **0.49 (0.27, 0.98)** |
| 1.04 (0.45, 2.47) | DAPT 1Mo+P2Y12 | 1.23 (0.35, 4.46) | 0.90 (0.27, 2.59) | 0.84 (0.32, 2.58) | 0.51 (0.19, 1.55) |
| 0.86 (0.34, 2.21) | 0.81 (0.22, 2.85) | DAPT 3Mo+ASA | 0.74 (0.21, 2.36) | 0.70 (0.24, 2.23) | 0.42 (0.14, 1.37) |
| 1.16 (0.62, 2.50) | 1.11 (0.39, 3.64) | 1.35 (0.42, 4.74) | DAPT 3Mo+P2Y12 | 0.95 (0.42, 2.75) | 0.57 (0.24, 1.65) |
| 1.22 (0.63, 2.16) | 1.19 (0.39, 3.13) | 1.43 (0.45, 4.21) | 1.05 (0.36, 2.41) | DAPT 6Mo | 0.61 (0.35, 1.03) |
| **2.03 (1.02, 3.66)** | 1.95 (0.65, 5.38) | 2.37 (0.73, 7.09) | 1.75 (0.61, 4.09) | 1.65 (0.97, 2.87) | L-DAPT |

**Major bleeding**

| | | | | | |
|---|---|---|---|---|---|
| DAPT 12Mo | 0.51 (0.14, 1.71) | 0.81 (0.22, 2.88) | 0.44 (0.18, 1.05) | 0.91 (0.29, 2.05) | 1.83 (0.65, 4.37) |
| 1.95 (0.59, 6.90) | DAPT 1Mo+P2Y12 | 1.59 (0.27, 9.33) | 0.87 (0.19, 3.92) | 1.80 (0.32, 6.92) | 3.61 (0.74, 16.18) |
| 1.23 (0.35, 4.51) | 0.63 (0.11, 3.77) | DAPT 3Mo+ASA | 0.54 (0.12, 2.57) | 1.14 (0.18, 4.52) | 2.21 (0.41, 10.06) |
| 2.27 (0.96, 5.48) | 1.15 (0.26, 5.20) | 1.85 (0.39, 8.56) | DAPT 3Mo+P2Y12 | 2.06 (0.48, 6.29) | **4.20 (1.08, 13.91)** |
| 1.10 (0.49, 3.43) | 0.56 (0.14, 3.17) | 0.88 (0.22, 5.43) | 0.49 (0.16, 2.07) | DAPT 6Mo | 2.01 (0.97, 5.15) |

| | | | | | |
|---|---|---|---|---|---|
| 0.55 (0.23, 1.54) | 0.28 (0.06, 1.35) | 0.45 (0.10, 2.41) | **0.24 (0.07, 0.93)** | 0.50 (0.19, 1.03) | L-DAPT |
| **Any bleeding** | | | | | |
| DAPT 12Mo | 0.63 (0.30, 1.36) | 0.81 (0.33, 1.92) | **0.52 (0.31, 0.92)** | 0.87 (0.43, 1.51) | 1.72 (0.89, 3.02) |
| 1.58 (0.74, 3.37) | DAPT 1Mo+P2Y12 | 1.28 (0.40, 4.03) | 0.83 (0.33, 2.16) | 1.41 (0.46, 3.33) | 2.72 (0.94, 6.80) |
| 1.23 (0.52, 3.01) | 0.78 (0.25, 2.50) | DAPT 3Mo+ASA | 0.65 (0.24, 1.85) | 1.09 (0.34, 2.95) | 2.12 (0.68, 5.95) |
| **1.91 (1.09, 3.19)** | 1.20 (0.46, 2.99) | 1.54 (0.54, 4.17) | DAPT 3Mo+P2Y12 | 1.68 (0.65, 3.42) | **3.28 (1.34, 6.88)** |
| 1.15 (0.66, 2.32) | 0.71 (0.30, 2.16) | 0.92 (0.34, 2.98) | 0.60 (0.29, 1.54) | DAPT 6Mo | **1.95 (1.25, 3.37)** |
| 0.58 (0.33, 1.12) | 0.37 (0.15, 1.07) | 0.47 (0.17, 1.47) | **0.31 (0.15, 0.75)** | **0.51 (0.30, 0.80)** | L-DAPT |
| **Definite or probable stent thrombosis** | | | | | |
| DAPT 12Mo | 1.18 (0.28, 5.45) | 2.06 (0.51, 8.65) | 0.80 (0.31, 2.27) | 0.73 (0.33, 1.71) | **0.40 (0.16, 0.95)** |
| 0.85 (0.18, 3.63) | DAPT 1Mo+P2Y12 | 1.67 (0.23, 12.83) | 0.66 (0.11, 4.32) | 0.61 (0.10, 3.30) | 0.34 (0.06, 1.84) |
| 0.49 (0.12, 1.96) | 0.60 (0.08, 4.38) | DAPT 3Mo+ASA | 0.40 (0.07, 2.22) | 0.35 (0.07, 1.91) | 0.19 (0.04, 1.03) |
| 1.24 (0.44, 3.18) | 1.52 (0.23, 8.72) | 2.51 (0.45, 13.71) | DAPT 3Mo+P2Y12 | 0.90 (0.26, 3.20) | 0.49 (0.13, 1.75) |
| 1.38 (0.59, 3.00) | 1.63 (0.30, 9.63) | 2.85 (0.52, 14.22) | 1.11 (0.31, 3.88) | DAPT 6Mo | 0.54 (0.26, 1.05) |
| **2.51 (1.05, 6.12)** | 2.95 (0.54, 17.88) | 5.21 (0.97, 26.99) | 2.04 (0.57, 7.72) | 1.84 (0.96, 3.80) | L-DAPT |
| **Net adverse clinical events** | | | | | |
| DAPT 12Mo | 0.92 (0.61, 1.41) | 0.95 (0.58, 1.54) | 0.65 (0.40, 1.07) | 0.83 (0.57, 1.28) | 0.93 (0.57, 1.61) |
| 1.09 (0.71, 1.64) | DAPT 1Mo+P2Y12 | 1.04 (0.54, 1.96) | 0.70 (0.37, 1.33) | 0.92 (0.52, 1.62) | 1.01 (0.54, 1.98) |
| 1.05 (0.65, 1.72) | 0.96 (0.51, 1.84) | DAPT 3Mo+ASA | 0.67 (0.34, 1.35) | 0.88 (0.48, 1.65) | 0.98 (0.50, 2.00) |
| 1.54 (0.94, 2.48) | 1.42 (0.75, 2.69) | 1.48 (0.74, 2.92) | DAPT 3Mo+P2Y12 | 1.30 (0.71, 2.42) | 1.45 (0.73, 2.90) |
| 1.20 (0.78, 1.76) | 1.09 (0.62, 1.91) | 1.14 (0.60, 2.09) | 0.77 (0.41, 1.41) | DAPT 6Mo | 1.11 (0.83, 1.51) |
| 1.07 (0.62, 1.76) | 0.99 (0.51, 1.85) | 1.02 (0.50, 1.99) | 0.69 (0.34, 1.37) | 0.90 (0.66, 1.20) | L-DAPT |

**Supplementary Table S13 The number of participants with newer-generation drug-eluting stents who had each outcome**

| Trial | Treatment | Total | All-cause mortality | MI | Major bleeding | Any bleeding | Definite or probable ST | NACE |
|---|---|---|---|---|---|---|---|---|
| TICO | DAPT 3Mo+P2Y12 | 1527 | 16 | 6 | 25 | 53 | 6 | 59 |
| | DAPT 12Mo | 1529 | 23 | 11 | 45 | 83 | 4 | 89 |
| STOPDAPT-2 | DAPT 1Mo+P2Y12 | 1500 | 21 | 13 | 3 | 6 | 4 | 35 |
| | DAPT 12Mo | 1509 | 18 | 11 | 16 | 23 | 1 | 55 |
| TWILIGHT | DAPT 3Mo+P2Y12 | 3555 | 34 | 95 | 34 | 141 | 14 | NA |
| | DAPT 12Mo | 3564 | 45 | 95 | 69 | 250 | 19 | NA |
| SMART-CHOICE | DAPT 3Mo+P2Y12 | 1495 | 21 | 11 | 12 | 28 | 3 | 65 |
| | DAPT 12Mo | 1498 | 18 | 17 | 14 | 49 | 2 | 81 |
| REDUCE | DAPT 3Mo+ASA | 733 | 23 | 26 | 24 | 24 | 12 | 85 |
| | DAPT 12Mo | 727 | 16 | 22 | 29 | 29 | 6 | 88 |
| GLOBAL LEADERS | DAPT 1Mo+P2Y12 | 7980 | 224 | 248 | 163 | 529 | 64 | 616 |
| | DAPT 12Mo | 7988 | 253 | 250 | 169 | 532 | 64 | 653 |
| OPTIMA-C | DAPT 6Mo | 683 | 2 | 1 | 1 | 1 | 0 | NA |
| | DAPT 12Mo | 684 | 3 | 1 | 1 | 1 | 1 | NA |
| DAPT-STEMI | DAPT 6Mo | 433 | 3 | 8 | 1 | 3 | 3 | 21 |
| | DAPT 12Mo | 437 | 6 | 8 | 2 | 5 | 4 | 29 |

| | | | | | | | | |
|---|---|---|---|---|---|---|---|---|
| SMART-DATE | DAPT 6Mo | 1357 | 35 | 24 | 6 | 35 | 15 | 96 |
| | L-DAPT | 1355 | 39 | 10 | 10 | 51 | 10 | 99 |
| NIPPON | L-DAPT | 1653 | 7 | 1 | 12 | 12 | 1 | 24 |
| | DAPT 6Mo | 1654 | 16 | 4 | 11 | 11 | 2 | 34 |
| IVUS-XPL | DAPT 6Mo | 699 | 5 | 1 | 5 | 5 | 2 | 15 |
| | DAPT 12Mo | 701 | 10 | 0 | 7 | 7 | 2 | 14 |
| I-LOVE-IT 2 | DAPT 6Mo | 909 | 14 | 42 | 13 | 57 | 5 | 72 |
| | DAPT 12Mo | 920 | 17 | 37 | 7 | 60 | 2 | 67 |
| ITALIC | DAPT 6Mo | 926 | 11 | 12 | 0 | 6 | 6 | 32 |
| | L-DAPT | 924 | 20 | 9 | 4 | 10 | 3 | 34 |
| DAPT Study | L-DAPT | 2345 | 49 | 48 | 21 | 57 | 6 | NA |
| | DAPT 12Mo | 2358 | 26 | 72 | 7 | 30 | 16 | NA |
| SECURITY | DAPT 6Mo | 682 | 14 | 21 | 5 | 6 | 3 | 46 |
| | DAPT 12Mo | 717 | 15 | 19 | 8 | 10 | 3 | 45 |
| OPTIMIZE | DAPT 3Mo+ASA | 1563 | 43 | 49 | 10 | 35 | 13 | 93 |
| | DAPT 12Mo | 1556 | 45 | 42 | 14 | 45 | 12 | 90 |
| PRODIGY | L-DAPT | 496 | 30 | 16 | NA | NA | 4 | NA |
| | DAPT 6Mo | 492 | 25 | 12 | NA | NA | 1 | NA |
| EXCELLENT | DAPT 6Mo | 540 | 3 | 9 | 2 | 3 | 3 | 16 |
| | DAPT 12Mo | 539 | 4 | 6 | 3 | 9 | 1 | 16 |

NA, the concrete outcomes was not available; MI, myocardial infarction; ST, stent thrombosis; ANCE, net adverse clinical events; DAPT, dual antiplatelet therapy; L-DAPT, longer than 12 months DAPT; DAPT 12Mo,12 months DAPT; DAPT 6Mo,6 months DAPT; DAPT 3Mo+ASA, 3-months DAPT followed by aspirin monotherapy; DAPT 3Mo+P2Y12, 3-month DAPT followed by a P2Y12 receptor inhibitor monotherapy; DAPT 1Mo+P2Y12,1-month DAPT followed by a P2Y12 receptor inhibitor monotherapy.

**Supplementary Table S14 Pooled estimates of the network meta-analysis of each endpoint in Participants with newer-generation drug-eluting stents**

**All-cause mortality**

| | | | | | |
|---|---|---|---|---|---|
| DAPT 12Mo | 0.94 (0.59, 1.64) | 1.10 (0.64, 1.96) | 0.83 (0.52, 1.34) | 0.94 (0.57, 1.45) | 1.20 (0.68, 1.88) |
| 1.07 (0.61, 1.71) | DAPT 1Mo+P2Y12 | 1.17 (0.53, 2.44) | 0.87 (0.43, 1.73) | 0.99 (0.47, 1.80) | 1.27 (0.55, 2.35) |
| 0.91 (0.51, 1.57) | 0.86 (0.41, 1.88) | DAPT 3Mo+ASA | 0.74 (0.36, 1.56) | 0.86 (0.39, 1.63) | 1.09 (0.47, 2.13) |
| 1.21 (0.75, 1.93) | 1.15 (0.58, 2.34) | 1.34 (0.64, 2.76) | DAPT 3Mo+P2Y12 | 1.13 (0.56, 2.07) | 1.45 (0.68, 2.73) |
| 1.06 (0.69, 1.74) | 1.01 (0.56, 2.15) | 1.16 (0.61, 2.59) | 0.88 (0.48, 1.78) | DAPT 6Mo | 1.27 (0.86, 1.87) |
| 0.83 (0.53, 1.47) | 0.79 (0.43, 1.81) | 0.91 (0.47, 2.14) | 0.69 (0.37, 1.47) | 0.79 (0.53, 1.16) | L-DAPT |

**Myocardial infarction**

| | | | | | |
|---|---|---|---|---|---|
| DAPT 12Mo | 1.01 (0.70, 1.55) | 1.16 (0.75, 1.83) | 0.87 (0.55, 1.23) | 1.14 (0.80, 1.64) | 0.71 (0.48, 1.08) |
| 0.99 (0.65, 1.43) | DAPT 1Mo+P2Y12 | 1.15 (0.62, 2.07) | 0.86 (0.44, 1.40) | 1.13 (0.66, 1.87) | 0.70 (0.40, 1.23) |
| 0.86 (0.55, 1.34) | 0.87 (0.48, 1.60) | DAPT 3Mo+ASA | 0.74 (0.39, 1.30) | 0.98 (0.54, 1.74) | 0.62 (0.33, 1.13) |
| 1.15 (0.82, 1.81) | 1.16 (0.72, 2.25) | 1.35 (0.77, 2.58) | DAPT 3Mo+P2Y12 | 1.30 (0.82, 2.38) | 0.83 (0.50, 1.57) |
| 0.88 (0.61, 1.25) | 0.89 (0.53, 1.51) | 1.02 (0.57, 1.86) | 0.77 (0.42, 1.22) | DAPT 6Mo | **0.63 (0.42, 0.93)** |
| 1.40 (0.92, 2.07) | 1.42 (0.82, 2.53) | 1.61 (0.89, 3.00) | 1.21 (0.64, 2.01) | **1.59 (1.07, 2.37)** | L-DAPT |

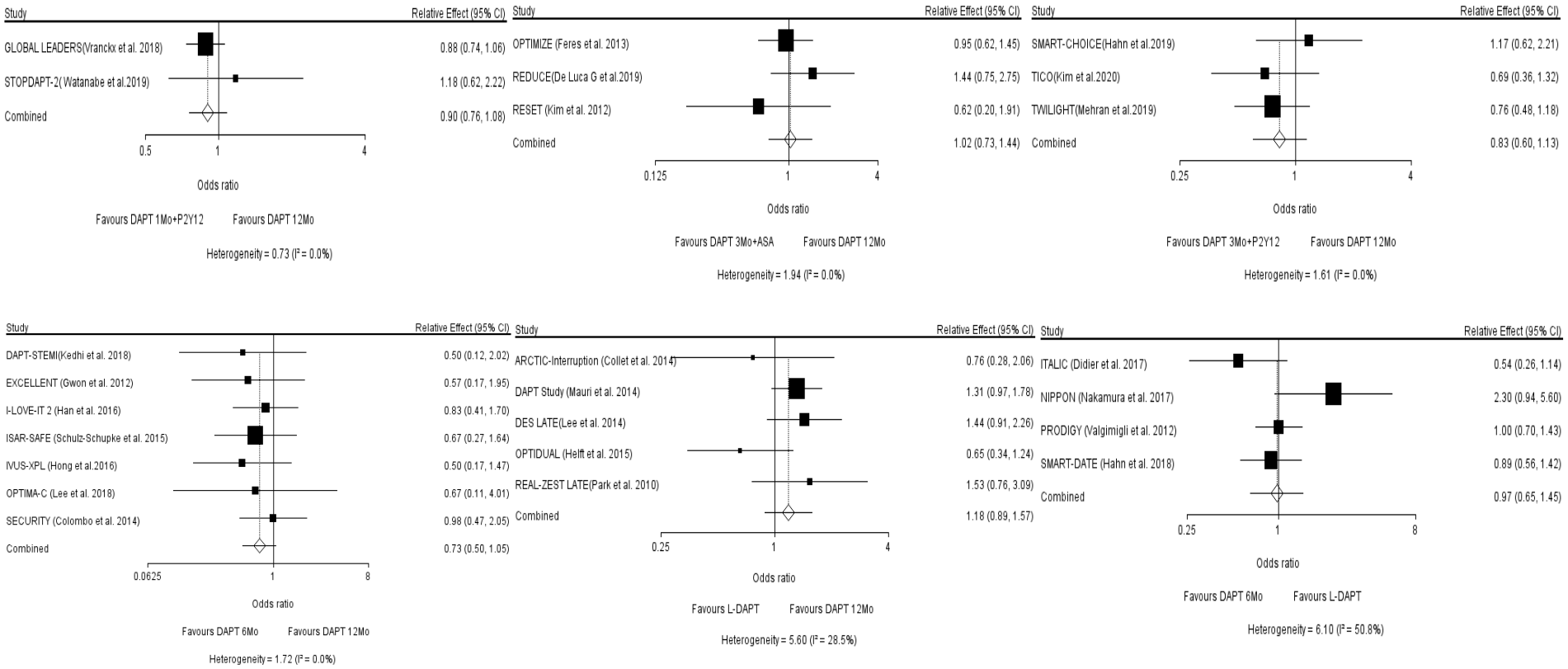
**Major bleeding**

| | | | | | |
|---|---|---|---|---|---|
| DAPT 12Mo | 0.65 (0.21, 1.31) | 0.78 (0.31, 1.84) | 0.58 (0.29, 1.21) | 1.05 (0.50, 1.98) | 2.13 (0.93, 5.37) |
| 1.54 (0.76, 4.69) | DAPT 1Mo+P2Y12 | 1.22 (0.41, 5.27) | 0.89 (0.35, 3.59) | 1.63 (0.59, 5.61) | **3.34 (1.15, 14.66)** |
| 1.28 (0.54, 3.23) | 0.82 (0.19, 2.47) | DAPT 3Mo+ASA | 0.74 (0.25, 2.47) | 1.34 (0.43, 4.09) | 2.75 (0.83, 10.16) |
| 1.73 (0.83, 3.39) | 1.12 (0.28, 2.85) | 1.35 (0.40, 4.04) | DAPT 3Mo+P2Y12 | 1.82 (0.62, 4.48) | **3.69 (1.22, 11.42)** |
| 0.95 (0.50, 1.98) | 0.61 (0.18, 1.68) | 0.74 (0.24, 2.31) | 0.55 (0.22, 1.60) | DAPT 6Mo | **2.04 (1.01, 5.02)** |

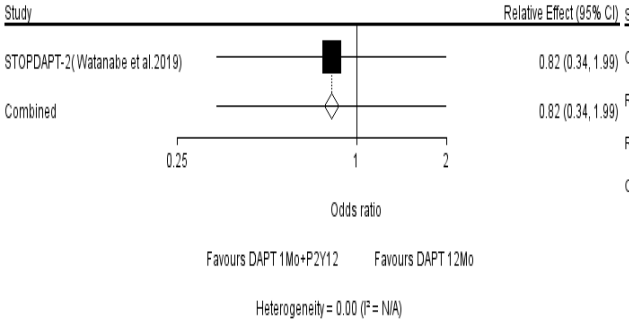
| | | | | | |
|---|---|---|---|---|---|
| 0.47 (0.19, 1.07) | **0.30 (0.07, 0.87)** | 0.36 (0.10, 1.20) | **0.27 (0.09, 0.82)** | **0.49 (0.20, 0.99)** | L-DAPT |
| **Any bleeding** | | | | | |
| DAPT 12Mo | 0.79 (0.37, 1.17) | 0.79 (0.45, 1.40) | **0.57 (0.38, 0.88)** | 0.85 (0.53, 1.25) | 1.42 (0.79, 2.30) |
| 1.26 (0.86, 2.74) | DAPT 1Mo+P2Y12 | 1.00 (0.52, 2.66) | 0.72 (0.43, 1.83) | 1.09 (0.61, 2.43) | 1.81 (0.95, 4.31) |
| 1.27 (0.72, 2.24) | 1.00 (0.38, 1.92) | DAPT 3Mo+ASA | 0.73 (0.36, 1.48) | 1.08 (0.49, 2.08) | 1.82 (0.78, 3.75) |
| **1.75 (1.14, 2.64)** | 1.39 (0.55, 2.30) | 1.38 (0.68, 2.79) | DAPT 3Mo+P2Y12 | 1.49 (0.78, 2.56) | **2.49 (1.20, 4.57)** |
| 1.18 (0.80, 1.88) | 0.92 (0.41, 1.63) | 0.93 (0.48, 2.02) | 0.67 (0.39, 1.29) | DAPT 6Mo | **1.67 (1.05, 2.68)** |
| 0.70 (0.43, 1.27) | 0.55 (0.23, 1.05) | 0.55 (0.27, 1.28) | **0.40 (0.22, 0.83)** | **0.60 (0.37, 0.95)** | L-DAPT |
| **Definite or probable stent thrombosis** | | | | | |
| DAPT 12Mo | 1.16 (0.57, 3.30) | 1.38 (0.60, 3.51) | 0.98 (0.46, 2.33) | 0.91 (0.47, 2.01) | 0.57 (0.25, 1.45) |
| 0.86 (0.30, 1.76) | DAPT 1Mo+P2Y12 | 1.17 (0.30, 3.60) | 0.83 (0.23, 2.57) | 0.78 (0.23, 2.28) | 0.49 (0.13, 1.56) |
| 0.72 (0.29, 1.68) | 0.85 (0.28, 3.39) | DAPT 3Mo+ASA | 0.71 (0.22, 2.42) | 0.69 (0.21, 2.18) | 0.41 (0.13, 1.52) |
| 1.02 (0.43, 2.18) | 1.20 (0.39, 4.37) | 1.40 (0.41, 4.58) | DAPT 3Mo+P2Y12 | 0.93 (0.33, 2.74) | 0.58 (0.19, 1.95) |
| 1.09 (0.50, 2.14) | 1.28 (0.44, 4.41) | 1.46 (0.46, 4.72) | 1.08 (0.36, 3.03) | DAPT 6Mo | 0.62 (0.31, 1.28) |
| 1.74 (0.69, 3.93) | 2.06 (0.64, 7.74) | 2.42 (0.66, 7.88) | 1.72 (0.51, 5.38) | 1.62 (0.78, 3.24) | L-DAPT |
| **Net adverse clinical events** | | | | | |
| DAPT 12Mo | 0.88 (0.62, 1.10) | 1.00 (0.72, 1.34) | **0.72 (0.52, 0.99)** | 1.00 (0.77, 1.29) | 0.95 (0.63, 1.39) |
| 1.14 (0.91, 1.61) | DAPT 1Mo+P2Y12 | 1.14 (0.77, 1.80) | 0.82 (0.57, 1.32) | 1.15 (0.81, 1.74) | 1.09 (0.69, 1.79) |
| 1.00 (0.74, 1.39) | 0.88 (0.56, 1.30) | DAPT 3Mo+ASA | 0.72 (0.47, 1.14) | 1.00 (0.67, 1.53) | 0.95 (0.58, 1.60) |
| **1.40 (1.01, 1.91)** | 1.22 (0.76, 1.76) | 1.38 (0.88, 2.14) | DAPT 3Mo+P2Y12 | 1.39 (0.92, 2.06) | 1.32 (0.80, 2.18) |
| 1.00 (0.78, 1.30) | 0.87 (0.58, 1.24) | 1.00 (0.65, 1.49) | 0.72 (0.49, 1.08) | DAPT 6Mo | 0.96 (0.70, 1.26) |
| 1.05 (0.72, 1.58) | 0.92 (0.56, 1.45) | 1.05 (0.63, 1.73) | 0.76 (0.46, 1.26) | 1.05 (0.79, 1.43) | L-DAPT |

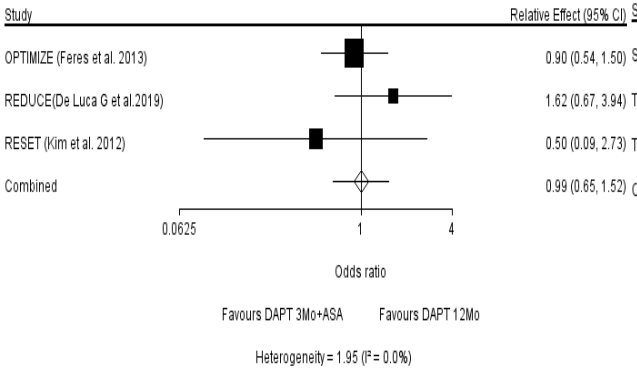
# Supplementary Fig. S1 Pair-wise meta-analysis of all endpoints

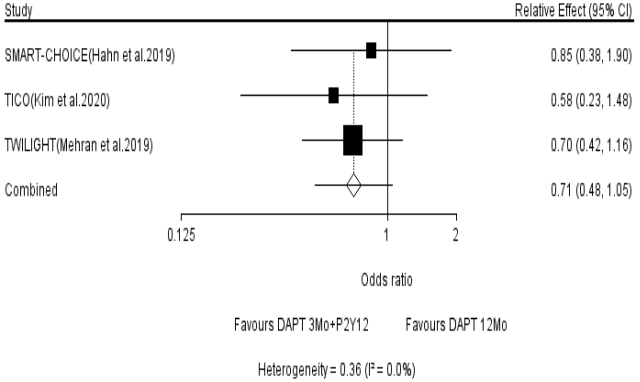
## All-cause mortality

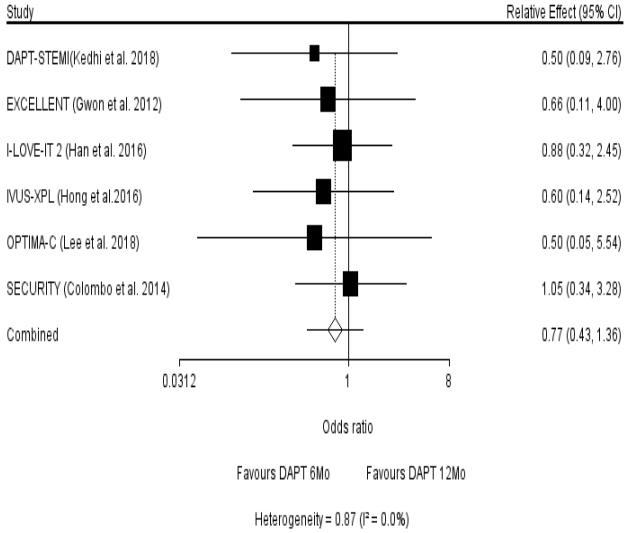


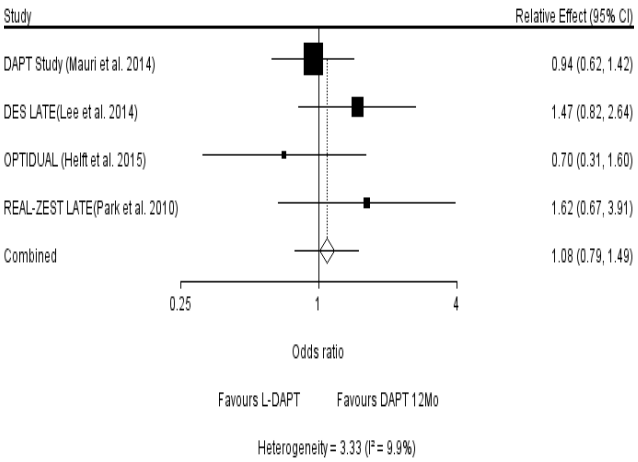
## Cardiac death

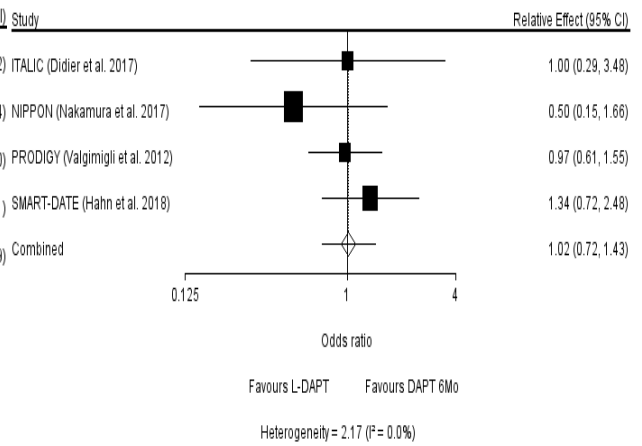


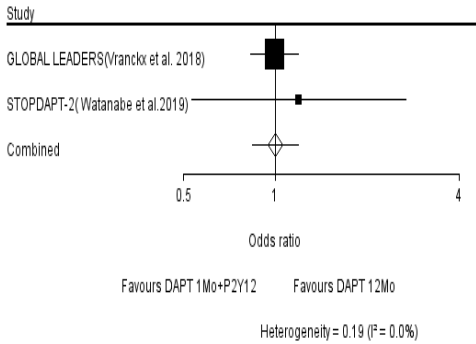



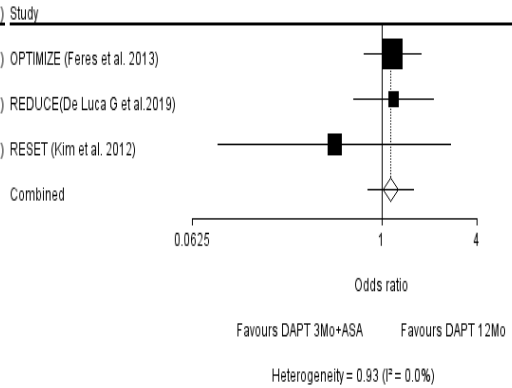



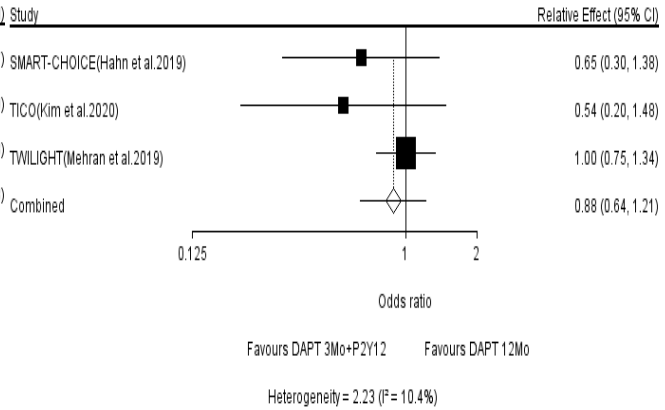



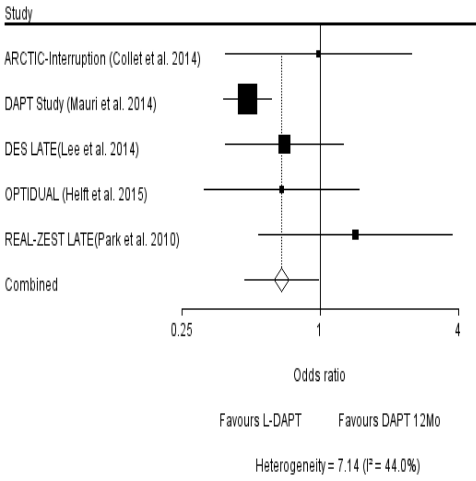



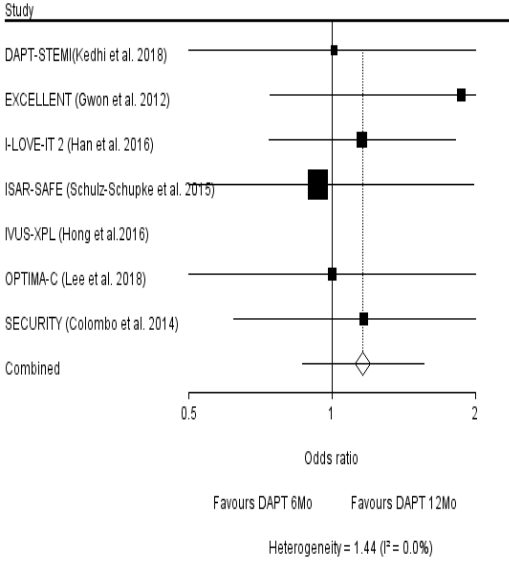
## Myocardial infarction

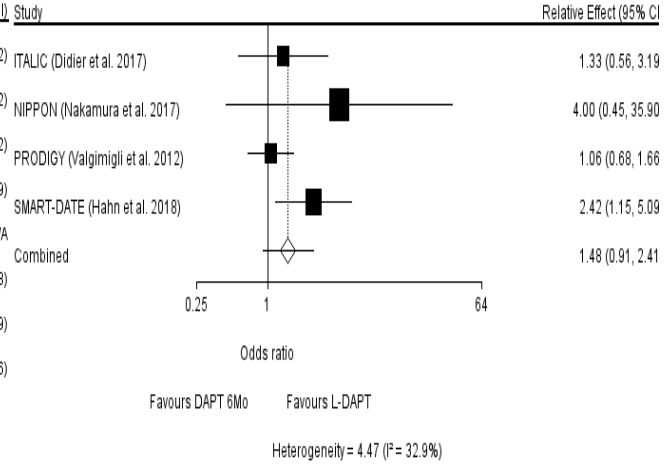


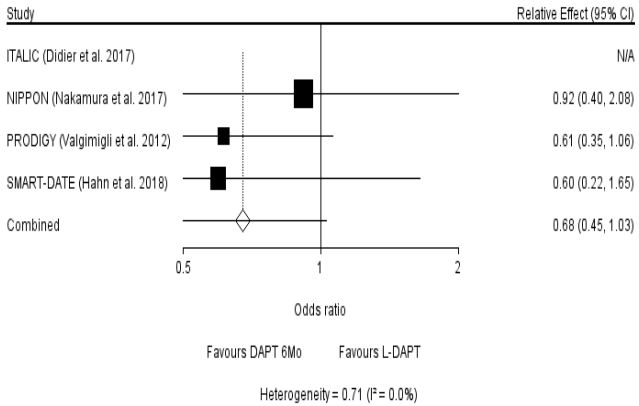



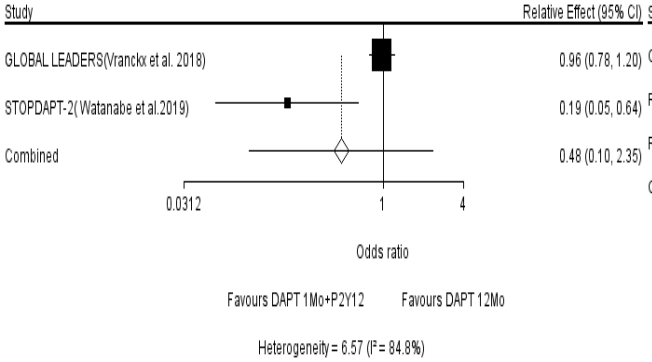



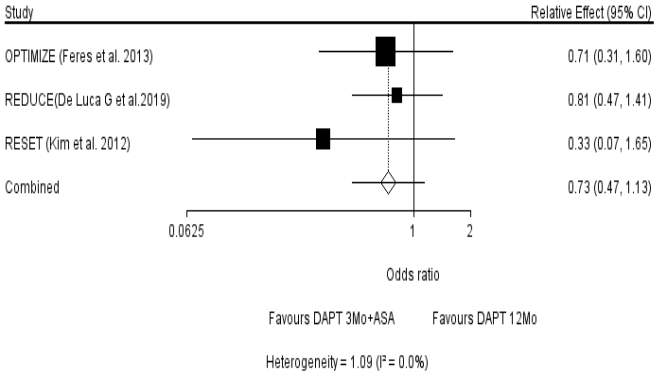



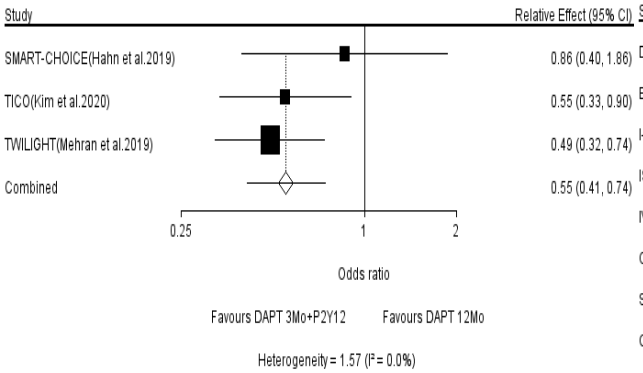



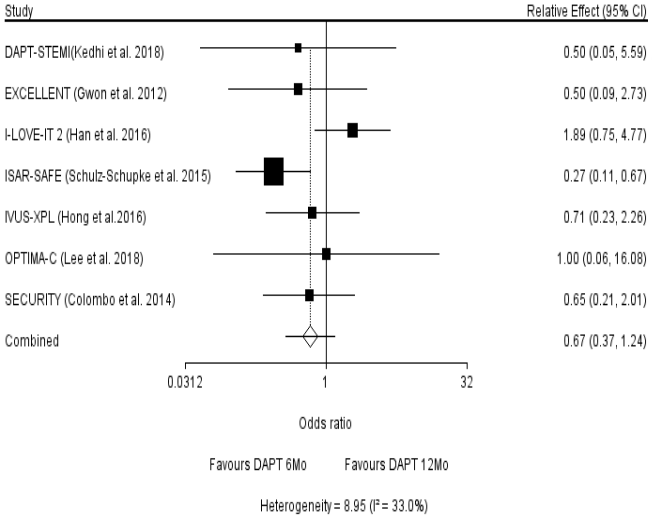
## Major bleeding

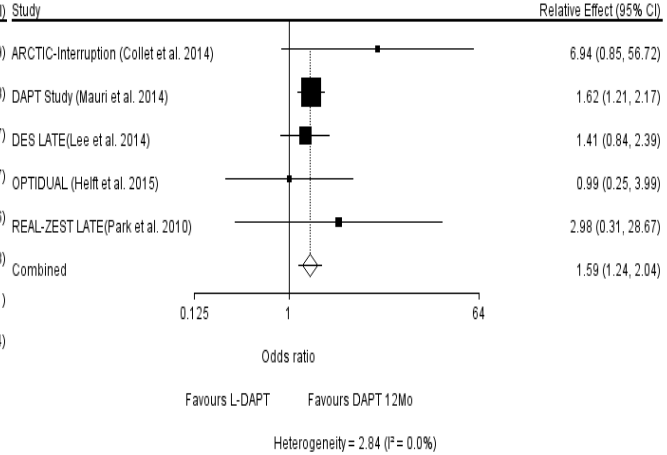


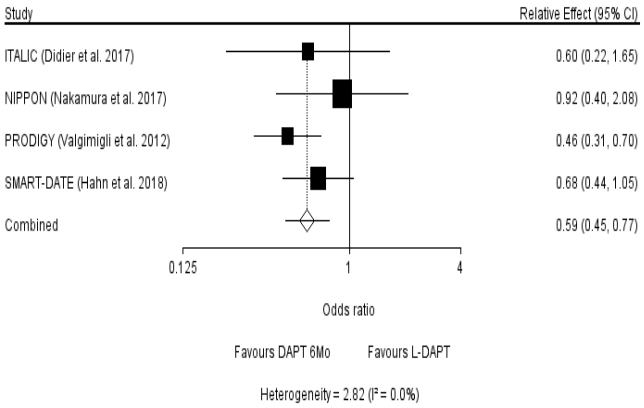



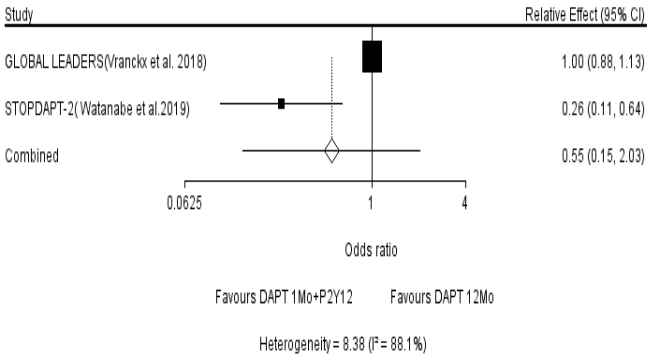



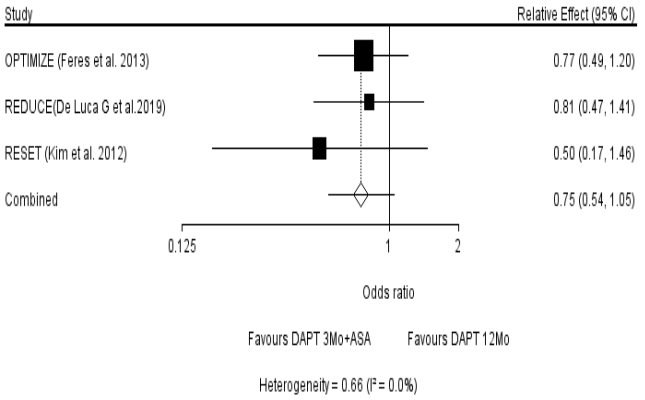



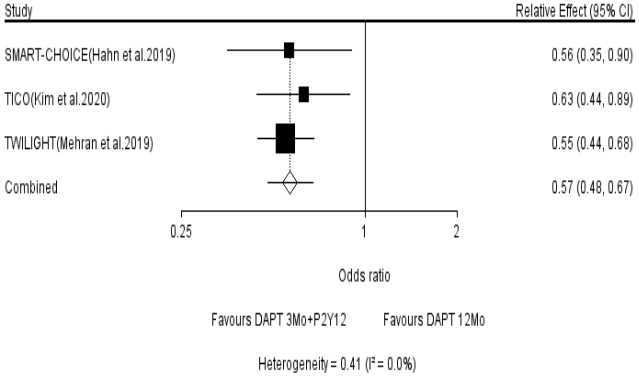



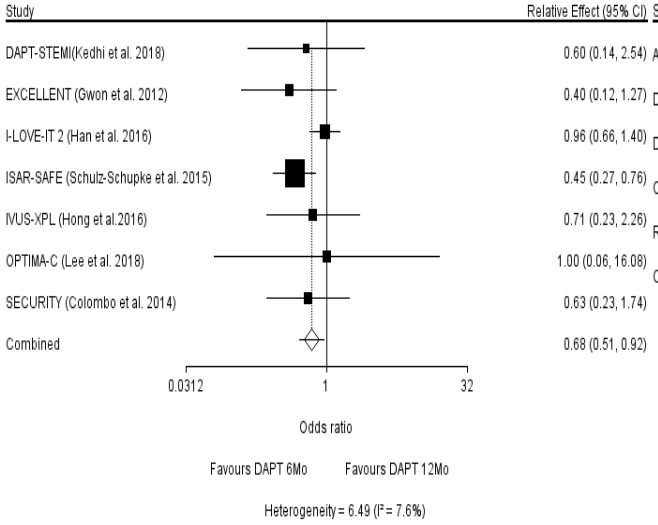
## Any bleeding

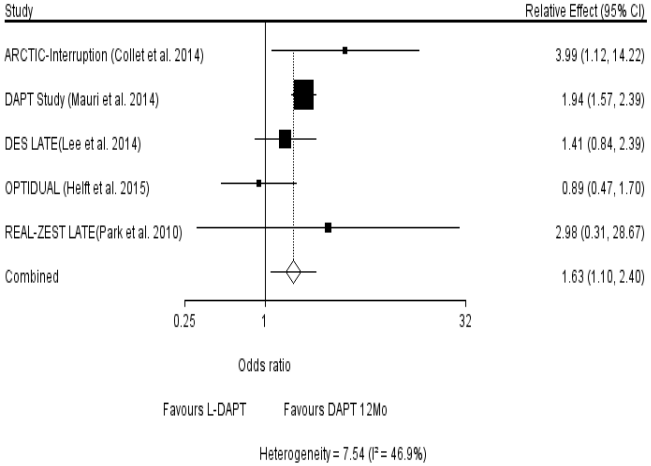


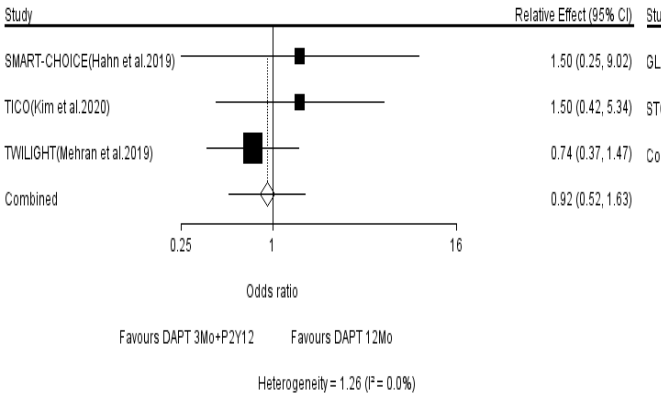



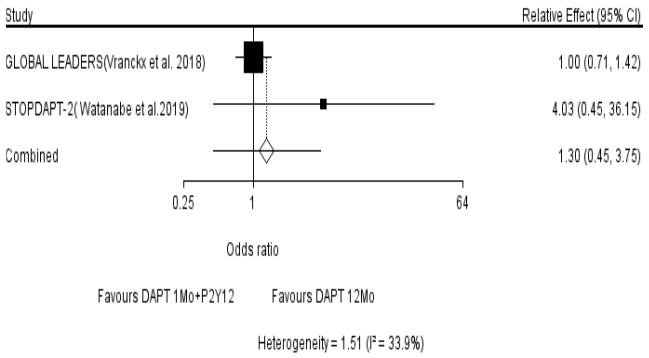



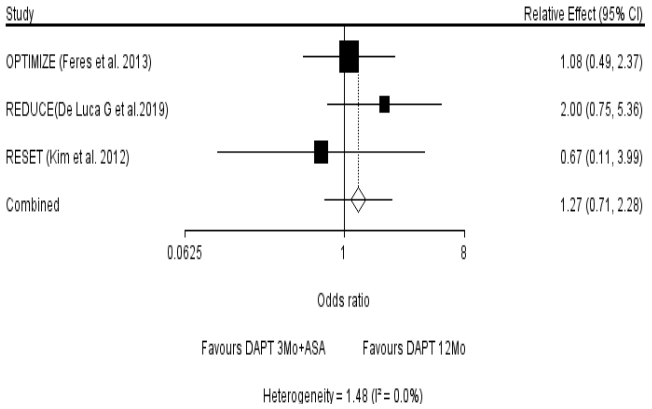



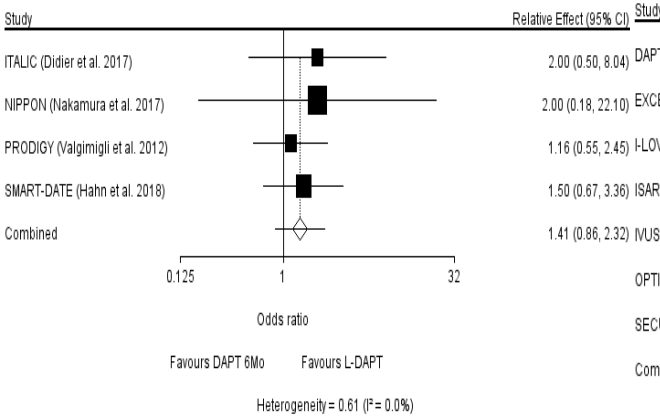



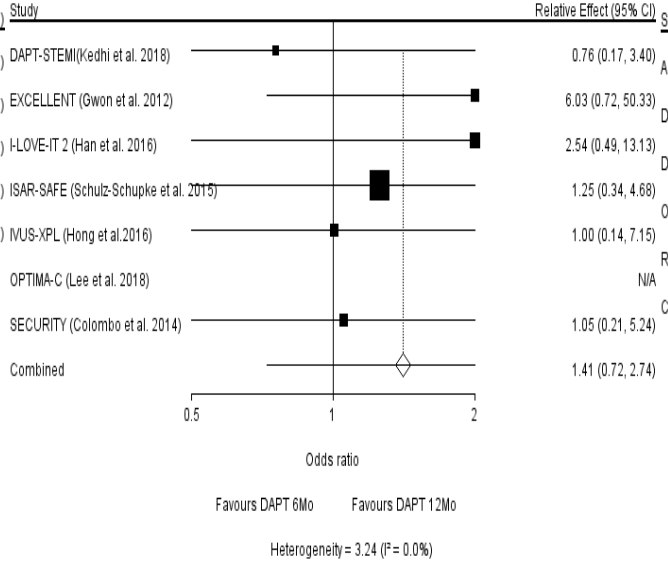
## Definite or probable stent thrombosis

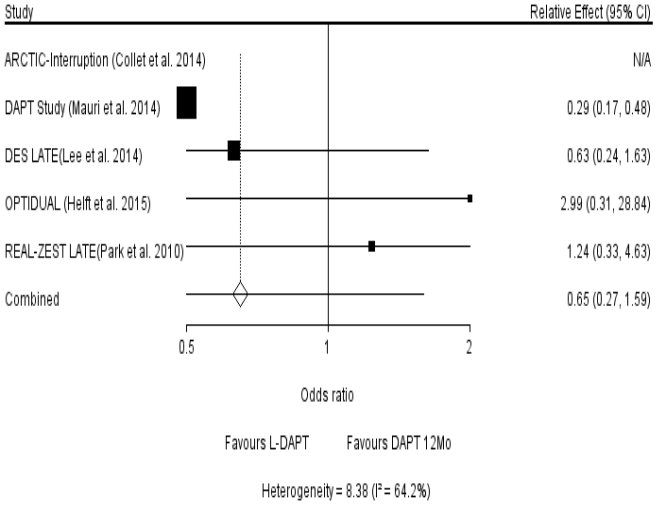


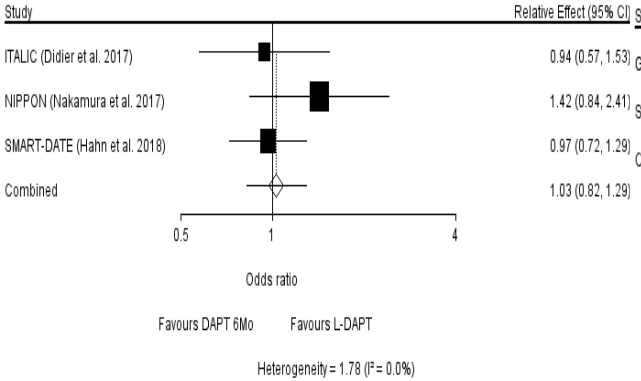



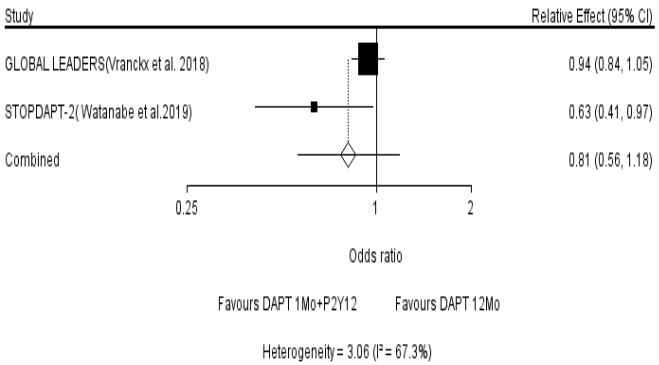



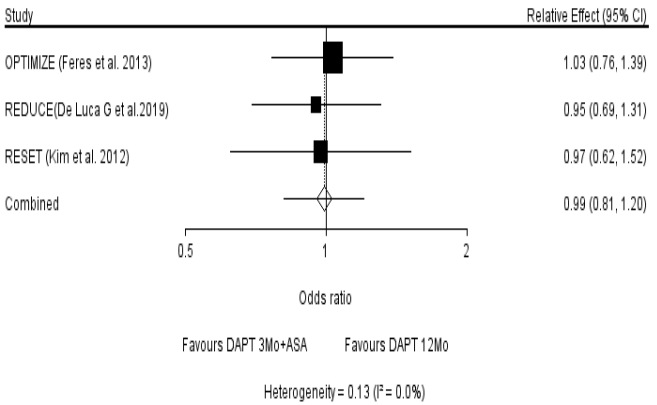



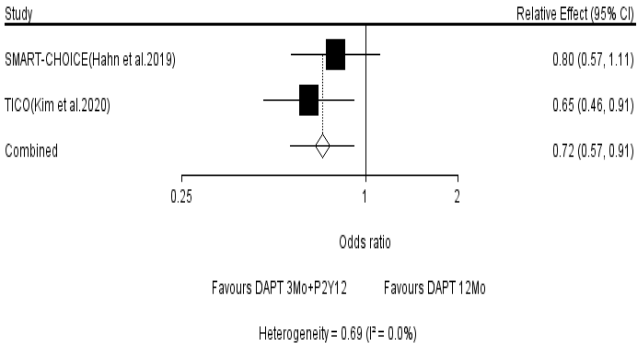



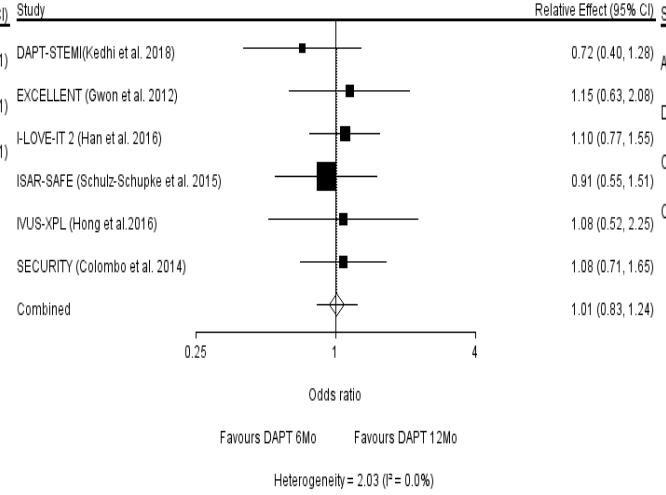
## Net adverse clinical events

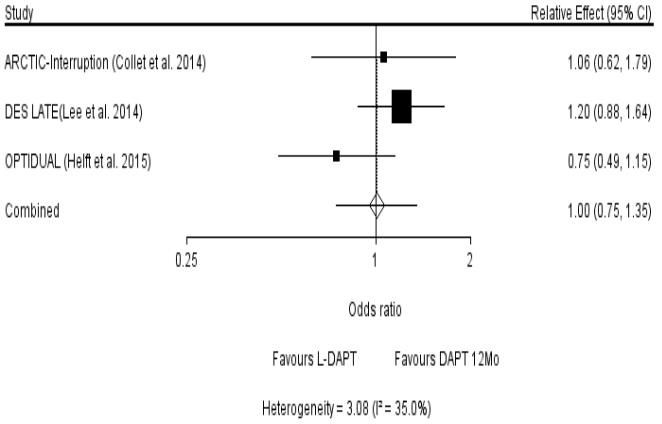
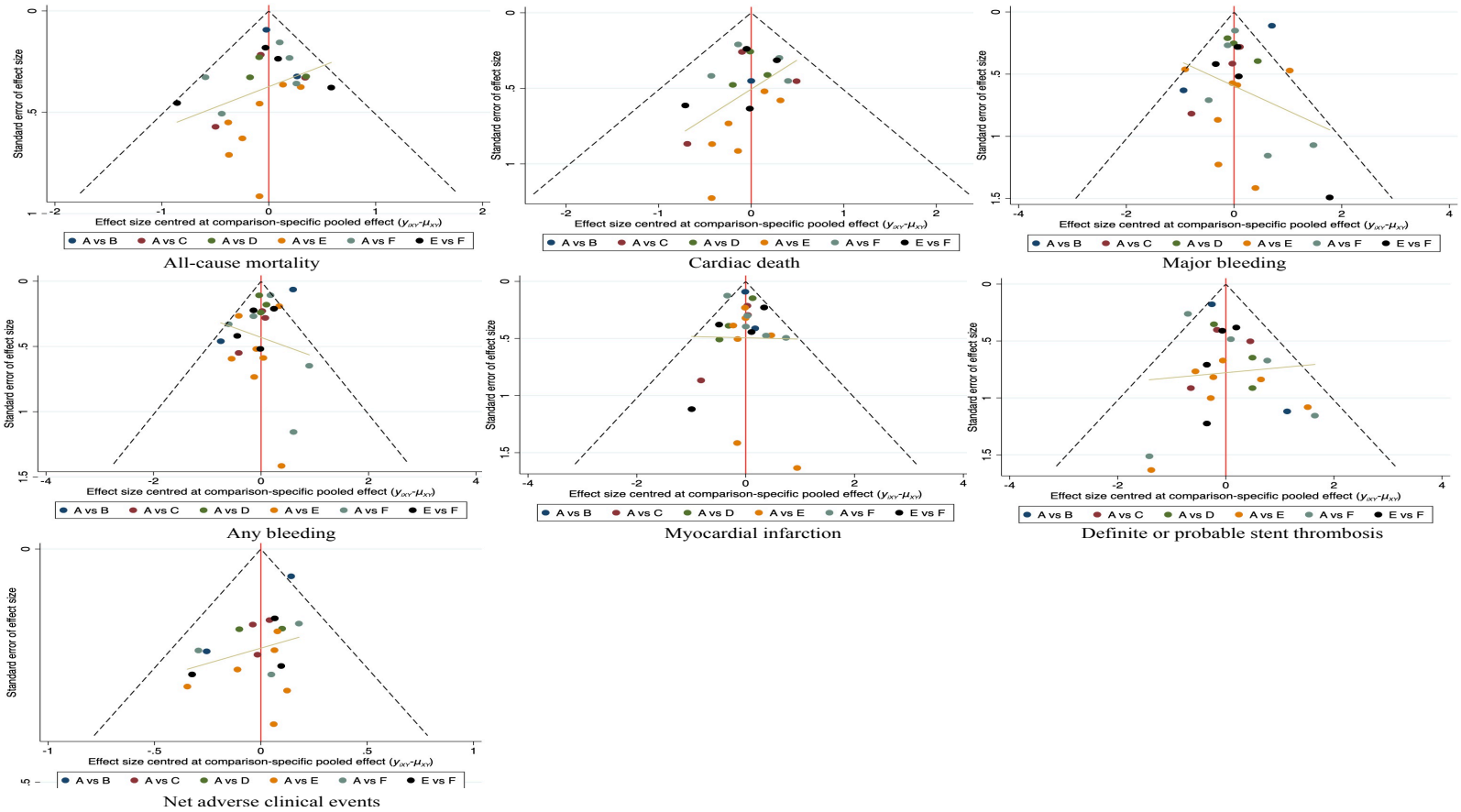





**Supplementary Fig. S2 Funnel plots of each endpoint.** A refers to DAPT 12Mo, B refers to DAPT 1Mo+P2Y12, C refers to DAPT 3Mo+ASA, D refers to DAPT 3Mo+P2Y12, E refers to DAPT 6Mo, and F refers to L-DAPT.